



BORDERLINE PERSONALITY DISORDER

THE NICE GUIDELINE ON TREATMENT AND MANAGEMENT

NATIONAL
COLLABORATING
CENTRE FOR
MENTAL HEALTH

TREATMENT AND MANAGEMENT

2.5.1 Current configuration of services

General adult mental health services in England and Wales offer varying levels of service provision for people with personality disorder. England and Wales have a health service in which personality disorder services are considered to be an integral part. As the decision to expand services to include the treatment of personality disorder was only made in 2003 the development of these services remains patchy and, in some areas, rudimentary. Although these services are for personality disorder generally, most users seeking services are likely to have a diagnosis of borderline personality disorder and this is anticipated in the service provision.

The programme in England includes the development of innovative psychosocial approaches to treatment, national service pilot projects and a workforce and training programme. The long-term plan is to develop capacity for specific personality services in all parts of the country.

2.5.2 Pharmacological treatment

Comorbid mental illness, particularly depression, bipolar disorder, PTSD, substance misuse disorder and psychosis are more common in people with borderline personality disorder than in the general population; lifetime prevalence of at least one comorbid mental illness approaches 100% for this group (Bender *et al.*, 2001). In addition, many of the trait- and state-related symptoms of borderline personality disorder (including affective instability, transient stress-related psychotic symptoms, suicidal and self-harming behaviours, and impulsivity) are similar in quality to those of many types of mental illness and could intuitively be expected to respond to drug treatment.

The use of antidepressants, mood stabilisers and antipsychotics is common in clinical practice. One large study of prescribing practice in the US found that 10% of people with borderline personality disorder had been prescribed an antipsychotic at some point during their contact with services, 27% a mood stabiliser, 35% an anxiolytic and 61% an antidepressant (Bender *et al.*, 2001); the lifetime prescribing rate for antidepressants was double that for patients with major depression. There are no published UK-based studies of prescribing practice, but given that people with borderline personality disorder tend to seek treatment, there is no reason to suspect that the prevalence of prescribing of psychotropic medication differs from that in the US. Such treatment is often initiated during periods of crisis and the placebo response rate in this context is high; the crisis is usually time limited and can be expected to resolve itself irrespective of drug treatment.

Often the prescribed drug is continued in an attempt to protect against further transient, stress-related symptoms and when these occur, another drug from a different class is likely to be added (Tyrer, 2002; Paris, 2002; Sanderson *et al.*, 2002). A

longitudinal study found that 75% of participants with borderline personality disorder were prescribed combinations of drugs at some point (Zanarini *et al.*, 2003). Those who have repeated crisis admissions to hospital may be prescribed multiple psychotropic drugs in combination with a range of medicines for minor physical complaints. Adherence to medication in the medium term is often poor and the frequency with which prescriptions are altered makes it difficult to see which drug, if any, has helped and how.

The psychotropic drugs that are commonly prescribed are all associated with clinically significant side effects. For example, antipsychotic drugs may lead to considerable weight gain (Theisen *et al.*, 2001), both compounding problems with self-esteem and increasing the risk of serious physical pathology such as diabetes and cardiovascular disease (Mackin *et al.*, 2005). Lithium can cause hypothyroidism and is a very toxic drug in overdose; valproate can lead to weight gain and is a major human teratogen (Wyszynski *et al.*, 2005); and selective serotonin re-uptake inhibitors (SSRIs) can cause unpleasant discontinuation symptoms if they are not taken consistently (Fava, 2006). The balance of risks and benefits of psychotropic drugs is generally even more unfavourable in adolescents and young adults: the risks associated with SSRIs, which have been associated with treatment-emergent suicidal ideation in young people (Hammad *et al.*, 2006), may outweigh the benefits (Whittington *et al.*, 2004), and valproate may increase the risk of young women developing polycystic ovaries (NICE, 2006a; NICE, 2007a).

No psychotropic drug is specifically licensed for the management of borderline personality disorder, although some have broad product licences that cover individual symptoms or symptom clusters. Where there is a diagnosis of comorbid depression, psychosis or bipolar disorder, the use of antidepressants, antipsychotics and mood stabilisers respectively would be within their licensed indications. Where there are depressive or psychotic symptoms, or affective instability, that fall short of diagnostic criteria for mental illness, the use of psychotropic drugs is largely unlicensed or 'off-label'. Prescribing off-label places additional responsibilities on the prescriber and may increase liability if there are adverse effects (Baldwin, 2007). As a minimum, off-label prescribing should be consistent with a respected body of medical opinion (Bolam test) and be able to withstand logical analysis (House of Lords, 1997). The Royal College of Psychiatrists recommends that the patient be informed that the drug prescribed is not licensed for the indication it is being used for, and the reason for use and potential side effects fully explained (Baldwin, 2007).

2.5.3 Psychological interventions

The history of specific psychological interventions designed to help people with borderline personality disorder is intertwined with changing conceptions of the nature of the disorder itself. The emergent psychoanalytic concept of 'borderline personality organisation', intermediate between neurosis and psychosis (Stern, 1938; Kernberg,

1967), was influential in the introduction of borderline personality disorder into DSM-III in 1980, but was not an approach taken by ICD-10. The borderline personality disorder concept was therefore first adopted in the US and had no wide currency in the UK before the mid-1980s. At this time, although a range of psychodynamic, experiential, behavioural and cognitive behavioural therapies were available within NHS mental health services, they were very patchy and in short supply. Cognitive therapy (CT) for depression was only in the early stages of being adopted. Many people who would now be described in terms of having borderline personality disorder presented with depression, anxiety and interpersonal difficulties and were offered these therapies. This spurred innovation as practitioners began to modify these techniques in order to help people with more complex psychological difficulties, and during the 1980s and 1990s systematic methods were developed specifically for this client group.

Specific therapies for borderline personality disorder, therefore, developed through modification of existing techniques. In both the US and UK, psychoanalytic methods were adapted to provide more structure, containment (such as explicit contracts between therapist and client) and responsiveness; for example, the classical technique of the 'blank screen' of therapist neutrality and abstinence was modified so that the therapist became more active. Derived (but distinct) from classical analytic technique, an approach based on developmental attachment theory led to a specific therapy emphasising mentalisation. A behavioural approach to self-harm and suicidality that incorporated skills training in emotion regulation and validation of client experience developed into dialectical behaviour therapy (DBT), a specific intervention for borderline personality disorder *per se*. Cognitive analytic therapy (CAT), which had from its outset explicitly addressed interpersonal difficulties, gained greater application to borderline problems through theoretical and practical attention to partially dissociated states of mind and their functional analysis. CT for depression was also adapted to personality disorders. For example, one method paid greater attention to the early maladaptive schemas underpinning cognitive biases. Adaptations have also been made in cognitive behavioural therapy (CBT) and interpersonal therapy (IPT). Some of these adapted therapies are offered as psychological therapy programmes (for example, mentalisation-based partial hospitalisation and DBT); others are provided as more straightforward time-limited one-to-one or group treatments (for example, CBT or CAT).

Despite the developments of these specific psychological therapies (see Chapter 5), most 'talking treatments' offered to people with borderline personality disorder in the NHS are generic or eclectic and do not use a specific method. Clinical psychologists are trained to work flexibly around a range of assessment, treatment and rehabilitation needs, through psychological formulation, treatment planning, staff supervision and environmental change. The British Psychological Society requires chartered clinical and counselling psychologists to train in two evidence-based psychological therapies, with further post-qualification training required before they can register as practitioners. However, they may not use a specific approach during

therapy sessions and, where a specific approach is used, it may not be available in the optimum format, that is, the one that was tested in clinical trials. A good example is DBT, which is a psychological therapy programme delivered by a team of therapists that includes one-to-one therapy sessions, psychoeducational groups and telephone support. Although NHS therapists may have trained in the method, it has proved organisationally difficult to ensure all elements of the DBT approach are available in practice.

Psychological and psychosocial interventions are delivered in a variety of ways and settings within the NHS by clinical psychologists and other staff trained in psychological therapies, such as psychiatrists, nurses, social workers and other mental health therapists. Individual and group therapies are available in psychology and psychotherapy departments, within day services and community mental health services. Day services have been established with specific expertise in programmes for this client group, some based on therapeutic community principles, but these are not universally available. In 2005, 11 pilot services were funded to demonstrate a range of service possibilities. All of these specified some element of psychological care, although few were based on provision of specific and formal psychological therapies (Crawford *et al.*, 2007).

In practice, the limiting factor in providing access to psychological therapies is the very small proportion of NHS staff trained to deliver these to a competent standard. A further challenge is how to embed psychological treatment into the overall care programme in health and social care, which may involve liaison among staff from many agencies who do not share a psychological understanding of the nature of the disorder. To address this, a psychological therapies framework can be applied to the care programme through multidisciplinary team-based training (Sampson *et al.*, 2006; Kerr *et al.*, 2007).

Together with greater understanding of the developmental origins and psychological mechanisms underpinning this disorder and epidemiological evidence on its natural history, the emergence of at least partially effective psychological treatments has challenged traditional views of borderline personality disorder as immutable. The therapeutic nihilism so characteristic of earlier decades is giving way to a belief that psychological therapies have an important role to play in the overall care, treatment and recovery of people with these disorders.

2.5.4 Arts therapies

Arts therapies developed mainly in the US and Europe. They have often been delivered as part of treatment programmes for people with personality disorders including those with borderline personality disorder. Arts therapies include art therapy, dance movement therapy, dramatherapy and music therapy which use arts media as its primary mode of communication; these four therapies are currently provided in the UK. Arts therapies are normally undertaken weekly, and a session lasts 1.5 to 2 hours. Patients are assessed for group (typically four to six members) or individual therapy. The primary concern is to effect change and growth through the use of the art form in

a safe and facilitating environment in the presence of a therapist. Arts therapies can help those who find it hard to express thoughts and feelings verbally. Traditionally, art therapy is thought of as working with primitive emotional material that is 'preverbal' in nature, and thus made available to exploration and rational thought. The nature of the therapist's work can thus be similar to the interpretations of psychoanalysis, or less interpretative and more supportive, to enable patients to understand what they want to understand from the work. For people with more severe borderline personality disorder, it is generally accepted that 'plunging interpretations' without sufficient support are unlikely to be helpful (Meares & Hobson, 1977).

Arts therapies are more concerned with the process of creating something, and the emotional response to this and/or the group dynamics of this. This can be very active (involving the physical characteristics of the art work and movement), playful, symbolic, metaphorical or lead directly to emotions that need to be understood. Such understanding may be achieved through subsequent discussion, and the use of the art materials when helpful.

2.5.5 Therapeutic communities

A therapeutic community is a consciously designed social environment and programme within a residential or day unit in which the social and group process is harnessed with therapeutic intent. In the therapeutic community the community itself is the primary therapeutic instrument (Kennard & Haigh, 2009).

In England therapeutic communities first emerged in a form that we would recognise today during the Second World War, at Northfield Military Hospital in Birmingham and Mill Hill in London. The leaders of the Northfield 'experiments' were psychoanalysts who were later involved in treatment programmes at the Tavistock Clinic and the Cassel Hospital, and had considerable international influence on psychoanalysis and group therapy. The Mill Hill programme, for battleshocked soldiers, later led to the founding of Henderson Hospital and a worldwide 'social psychiatry' movement, which brought considerably more psychological and less custodial treatment of patients of mental hospitals throughout the Western world.

Different forms of therapeutic community have evolved from these origins, one clear strand of which is for specific treatment of people with personality disorders. The therapeutic communities for personality disorder range from full-time residential hospitals to units that operate for a few hours on one day each week. Although, as stated above, the community itself is the primary therapeutic agent, programmes include a range of different therapies, usually held in groups. These can include small analytic groups, median analytic groups, psychodrama, transactional analysis, arts therapies, CT, social problem solving, psychoeducation and gestalt. In addition to specific therapies, there are community meetings and activities.

Therapeutic communities generally use a complex admission procedure, rather than straightforward inclusion and exclusion criteria. This results in diagnostic

heterogeneity, and none claims to treat borderline personality disorder exclusively; however recent work has demonstrated that the admission characteristics of members show high levels of personality morbidity, with most exhibiting sufficient features to diagnose more than three personality disorders, often in more than one cluster. The admission phase includes engagement, assessment, preparation and selection processes before the definitive therapy programme begins and is a model of stepped care, where the service users decide when and whether to proceed to the next stage of the programme. A voting procedure by the existing members of the community, at a specifically convened case conference or admissions panel, is normally used to admit new members. Programmes and their various stages are time limited, and none of the therapeutic communities specifically for personality disorder is open ended. Some have formal or informal, staff or service-user led post-therapy programmes.

Staff teams in therapeutic communities are always multidisciplinary, drawn mostly from the mental health core professions, including direct psychiatric input and specialist psychotherapists. They also frequently employ 'social therapists', who are untrained staff with suitable personal characteristics, and ex-service users. The role of staff is less obvious than in single therapies, and can often cover a wide range of activities as part of the sociotherapy. However, clear structures – such as job descriptions defining their different responsibilities, mutually agreed processes for dealing with a range of day-to-day problems and rigorous supervisory arrangements – always underpin the various staff roles.

There are several theoretical models on which the clinical practice is based, drawing on systemic, psychodynamic, group analytic, cognitive-behavioural and humanistic traditions. The original therapeutic community model at Henderson Hospital was extensively researched in the 1950s using anthropological methods and four predominant 'themes' were identified: democratisation, permissiveness, reality confrontation and communalism. More contemporary theory emphasises the following: the role of attachment; the 'culture of enquiry' within which all behaviours, thinking and emotions can be scrutinised; the network of supportive and challenging relationships between members; and the empowering potential of members being made responsible for themselves and each other. This has been synthesised into a simple developmental model of emotional development, where the task of the therapeutic community is to recreate a network of close relationships, much like a family, in which deeply ingrained behavioural patterns, negative cognitions and adverse emotions can be re-learned.

For personality disorders, the non-residential communities are mostly within the NHS mainstream mental health services, and the residential units are in both NHS and tier 3 organisations. Standards have been devised to ensure uniformity and quality of practice, and all NHS therapeutic communities for personality disorder participate in an annual audit cycle of self-review, peer review and action planning against these standards. The Department of Health in England has supported the recent development of 'NHS commissioning standards' upon which accreditation for therapeutic communities will be based.

2.5.6 Other therapies

This section includes various modalities that are not part of the general psychological treatments for borderline personality disorder. Group analytic psychotherapy, humanistic and integrative psychotherapy and systemic therapy can all be routinely employed in work with people with personality disorder, either as stand-alone therapies for less complex cases or as part of multidisciplinary packages of care – or longterm pathways – for those with more intractable or severe conditions.

Group analytic psychotherapy

This is also often known simply as ‘group therapy’. It is characterised by non-directive groups (without pre-determined agendas), in which the relationships between the members, and the members and the therapist (‘conductor’), comprise the main therapeutic tool. Such groups generally, and deliberately, build a strong *esprit de corps* and are both strongly supportive and deeply challenging. The membership of a group is fairly constant, with each member staying typically for 2 to 5 years. Suitably qualified group therapists (to United Kingdom Council for Psychotherapy [UKCP] standards) undergo at least 4 years’ training, have regular clinical supervision and undertake continuing professional development (CPD) activities.

The group process can help prevent hazardous therapeutic relationships developing with a therapist, as can happen in individual therapy with people with severe personality disorders. They can actively address relationship difficulties that are manifest ‘live’ in the group, and they can avoid difficult dependency by helping participants to take responsibility for themselves by first sharing responsibility for each other and later learning how to ask for help for themselves, in an adaptive way.

Disadvantages include difficulty in initiating participation because of the fear of personal exposure; problems of finding a regular suitable meeting space; and issues of confidentiality.

Humanistic and integrative psychotherapies

These are therapies based on a variety of theoretical models that evolved in the mid-20th century as alternatives to the dominant model of psychoanalysis. There is a significant overlap with the term ‘action therapies’, which has increasing currency. They include: psychodrama, which is group-based and aims to understand particularly difficult past emotional episodes and link them to current problems and difficulties; transactional analysis, which is based on parent, adult and child ‘ego states’ (a person’s beliefs, mannerisms and emotional responses), and can be undertaken either individually or in groups; gestalt therapy, which aims to facilitate awareness and help achieve self-regulation and self-actualisation (therapeutic techniques include empty-chair work, role reversal and enactments); and person-centred therapy developed from Carl Rogers’ humanistic approach.

Systemic therapy

This is most commonly used for work with families (or support networks), for example, where the index patient is a child. It aims to maximise family strengths and resilience to help people overcome problems experienced by individual family members or the family as a whole. It helps family members to understand how they function as a family and to develop more helpful ways of interacting with and supporting each other. It uses a format with long but widely-spaced sessions, for example 2 hours every 6 weeks. It requires a supervising team who watch the session live or who listen to it with audio equipment, and who discuss hypotheses of how the system is working and actions to bring about change. The individual and family or support network have access to the ideas and hypotheses discussed in the team, so that different experiences and points of view can be heard and acknowledged. The therapists help the family (or support network) to bring about the changes that they have identified as therapeutic goals. There are a number of models of systemic theory and interventions, such as Milan, social constructionist, narrative, solution focused, structural and strategic. The interventions are generally 'structural' or 'strategic', and include the use of such techniques as circular questioning (for example, 'what would your brother think about your mother's answer to that question?'), reframing and mapping the system with genograms (a pictorial representation of a patient's family relationships).

In cases of personality disorder where the dynamics within a whole family may be important in maintaining or exacerbating the presenting range of problems, and the family members are willing to participate, systemic therapy can be effective at starting new ways of communicating within a family that may be self-sustaining.

Nidotherapy

Nidotherapy, from the Latin, *nidus*, meaning nest (Tyrer *et al.*, 2003a), is distinct from psychotherapeutic approaches in that the emphasis is on making environmental changes to create a better fit between the person and their environment. In this sense it is not specifically a treatment, but it does have a therapeutic aim of improving quality of life, through acceptance of a level of handicap and its environmental accommodation.

2.6 MULTI-AGENCY PERSPECTIVE

2.6.1 The NHS and personality disorder

The perceived enduring and chronic nature of personality disorder poses a challenge to a healthcare system that is historically, and to a large extent still is, strongly influenced by the biological (illness) paradigm of mental health. Essentially, mental health services within the NHS have been configured in such a way as to 'treat' people during the acute phases of their illness. As personality disorders by their definition do

not have ‘acute’ phases some have argued that a personality disorder should not be the responsibility of the NHS (see Kendell [2002] for further discussion).

Given the confusion that surrounds the nature of personality disorder, it is not surprising that this has impacted on NHS care for people with this diagnosis. Until recently, personality disorder services in the NHS had been diverse, spasmodic and inconsistent (Department of Health, 2003).

2.6.2 The National Service Framework (NSF) for Mental Health

In line with the NSF for Mental Health (Department of Health, 1999a) the National Institute for Mental Health in England (NIMHE) produced policy implementation guidance for the development of services for people with personality disorder (Department of Health, 2003). The main purpose of this document was:

- to assist people with personality disorder who experience significant distress or difficulty to access appropriate clinical care and management from specialist mental health services
- to ensure that offenders with a personality disorder receive appropriate care from forensic services and interventions designed both to provide treatment and to address their offending behaviour
- to establish the necessary education and training to equip mental health practitioners to provide effective assessment and management.’ (Department of Health, 2003). The Personality Disorder Capabilities Framework (NIMHE, 2003) soon followed. This document set out a framework to support the development of the skills that would enable practitioners to work more effectively with people with personality disorders. It also aimed to provide a framework to support local and regional partners to deliver appropriate education and training (NIMHE, 2003). This document did not focus solely on the needs of NHS organisations; it had a wider remit to include all agencies that had contact with people who met the diagnosis. These two documents, along with investments in pilot personality disorder services and training initiatives, have signalled a significant change in the perspective of the NHS on personality disorder and have led to its commitment to enhance and improve its service.

2.6.3 Social services

The role of social services, in providing care and support to people with mental health problems, covers a wide range of people, from those with mild mental health problems to people with severe and enduring mental disorders (Department of Health, 1998). Historically, care provided by social services is determined by the person’s social need and is less influenced by diagnosis and the biological paradigm than the NHS. After the 1998 White Paper on modernising social services (Department of Health, 1998),

which aimed to set new standards of performance and to allow the NHS and social services to have closer partnerships in meeting the standards set down in the NSF for mental health, local implementation teams were set up across the country. With respect to personality disorder, their role is to review the progress that local mental health and social care services are making towards implementing the NSF's targets for personality disorder.

2.6.4 Criminal justice system

In law, personality disorder is generally seen as distinct from 'serious mental illness' because it is not considered to reduce the person's capacity to make decisions (Hart, 2001). Instead, it is thought of as an aggravating condition (Hart, 2001). Nevertheless, new legislation in the Mental Health Act amendment (HMSO, 2007) and the Mental Capacity Act (HMSO, 2005) will change both the rights and protections for people with personality disorders and their access to services. However, the legal position that people with personality disorder have held throughout the history of psychiatry has undoubtedly influenced the perspective of the criminal justice system regarding personality disorder and goes some way to explain why most people with personality disorder would generally find themselves in the criminal justice system as opposed to forensic mental health services. It is not uncommon within forensic mental health services for regional secure units to actively exclude patients with a primary diagnosis of personality disorder, because they do not consider this to be their core business (Department of Health, 2003). In many parts of the country there are no specific services, and, when services are offered, they tend to be idiosyncratic.

In March 1999, a report commissioned by the Department of Health about the future organisation of prison healthcare (Department of Health, 1999b) proposed that people in prison should have access to the same quality and range of services (including mental health) as the general public (Department of Health, 1999b). In the same year the NSF called for closer partnerships between prisons and the NHS at local, regional and national levels (Department of Health, 1999a). The emphasis was on a move towards the NHS taking more responsibility for providing mental healthcare in prisons and establishing formal partnerships.

In July 1998, the Secretary of State announced a review of the 1983 Mental Health Act (Department of Health, 1983), triggered by concerns that current legislation did not support a modern mental health service. These concerns were reiterated in the NSF for mental health since 'neither mental health nor criminal justice law currently provides a robust way of managing the small number of dangerous people with severe personality disorder' (Department of Health, 1999a).

2.7 YOUNG PEOPLE

Diagnosing borderline personality disorder in young people under 18 has often caused controversy. Although borderline personality disorder is thought to affect between 0.9 and 3% of the community population of under 18 year olds (Lewinsohn *et al.*, 1997; Bernstein *et al.*, 1993), there is some uncertainty about the rate (see Chapter 9). There are also certain caveats in DSM-IV and ICD-10 when making the diagnosis in young people (see Chapter 9). However young people with borderline personality disorder often present to services in seek of help (Chanen *et al.*, 2007a). Because interventions for young people with borderline personality disorder will usually be provided by specialist CAMHS, which has a different structure from adult mental health services, a full discussion of the issues relating to young people with borderline personality disorder can be found in Chapter 9.

2.8 THE EXPERIENCE OF SERVICE USERS, AND THEIR FAMILIES AND CARERS

There are particular issues for people with borderline personality disorder regarding the diagnosis, the label and associated stigma, which can have an impact on people accessing services and receiving the appropriate treatment. These issues are fully explored in Chapter 4, which comprises personal accounts from people with personality disorder and from a carer, and a review of the literature of service user and family/carer experience.

The families and carers of people may also feel unsupported in their role by healthcare professionals and excluded from the service user's treatment and care. The issues surrounding this are also further explored in Chapter 4. Although there are debates around the usefulness and applicability of the word 'carer', this guideline uses the term 'families/carers' to apply to all people who have regular close contact with the person and are involved in their care.

2.9 ECONOMIC IMPACT

Besides functional impairment and emotional distress, borderline personality disorder is also associated with significant financial costs to the healthcare system, social services and the wider society. The annual cost of personality disorders to the NHS was estimated at approximately £61.2 million in 1986 (Smith *et al.*, 1995). Of this, 91% accounted for inpatient care. Another study conducted in the UK, estimated the costs of people with personality disorders in contact with primary care services (Rendu *et al.*, 2002). The study reported that people with personality disorders incurred a cost of around £3,000 per person annually, consisting of healthcare costs and productivity losses; in contrast, the respective cost incurred by people without personality disorders

in contact with primary care services was £1,600 (1998/99 prices). In both groups, productivity losses accounted for over 80% of total costs. Dolan and colleagues (1996) assessed the cost of people with personality disorders admitted to a UK hospital over 1 year prior to admission; this cost was reported to reach £14,000 per person (1992/93 prices), including inpatient and outpatient healthcare costs, as well as prison-related costs (which amounted to approximately 10% of the total cost). Although the two UK studies (Rendu *et al.*, 2002; Dolan *et al.*, 1996) differed in methodology and costs considered, this difference in costs may be partly attributed to the different levels of severity of the disorders apparent in the two study populations (people engaged with general practice services versus people admitted to hospital).

The economic cost of personality disorders has been assessed in other European countries as well: in Germany, inpatient treatment of borderline personality disorder was estimated at €3.5 billion annually, covering about 25% of the total costs for psychiatric inpatient treatment in the country (Bohus, 2007). In the Netherlands, the average cost of a person with personality disorder referred for psychotherapeutic treatment was estimated at €11,000 (2005 prices) over 12 months prior to treatment (Soeteman *et al.*, 2008). Of this, 66.5% was associated with healthcare expenditure, while the rest reflected productivity losses. According to another study (Van Asselt *et al.*, 2007), the average cost per person with borderline personality disorder in the Netherlands was €17,000 in 2000. Of this, only 22% was health-related. The remaining cost was incurred by out-of-pocket expenses, informal care, criminal justice costs and productivity losses. Based on this average cost and a prevalence of borderline personality disorder of 1.1%, the study estimated that the total societal cost of borderline personality disorder in the Netherlands reached €2.2 billion in 2000. The authors noted that the direct medical costs represented only 0.63% of total Dutch healthcare expenditure in 2000, which meant that, given the 1.1% prevalence of the condition, people with borderline personality disorder seemed to use a less than proportionate share of the healthcare budget. However, the authors acknowledged that people in institutional care were not part of the study sample, and therefore medical costs associated with borderline personality disorder might have been underestimated.

Treatment-seeking people with personality disorders have been reported to place a high economic cost on society, compared with people with other mental disorders such as depression or generalised anxiety disorder (GAD) (Soeteman *et al.*, 2008). People with borderline personality disorder make extensive use of more intensive treatments, such as emergency department visits and psychiatric hospital services (Bender *et al.*, 2001 & 2006; Chiesa *et al.*, 2002), resulting in higher related healthcare costs compared with people with other personality disorders and major depression (Bender *et al.*, 2001 & 2006). In addition, they are more likely to use almost every type of psychosocial treatment (except self-help groups) and to have used most classes of medication compared with people with depression (Bender *et al.*, 2001). However, an American prospective study that followed people with borderline personality disorder over 6 years (Zanarini *et al.*, 2004a) reported that, although hospitalisation rates and rates of day or residential treatment were high at initiation of the study, these

significantly declined overtime; similar patterns were observed for rates of intensive psychotherapy, although engagement in less intensive psychosocial therapeutic programmes remained stable over the 6 years of the study. Polypharmacy was a characteristic of people with borderline personality disorder that was not affected by time, with 40% of people taking three or more concurrent standing medications, 20% taking four or more and 10% taking five or more, at any follow-up period examined. The authors concluded that the majority of people diagnosed with borderline personality disorder carry on outpatient treatment in the long term, but only a declining minority continue to use restrictive and more costly forms of treatment.

The level of severity of symptoms of borderline personality disorder determines the level of usage of healthcare resources: in a study conducted in a primary care setting in the US, the severity of symptoms experienced by women with borderline personality disorder was shown to predict increased use of primary healthcare resources (Sansone *et al.*, 1996). This finding was consistent with the findings of another American study that examined male veterans with borderline personality disorder (Black *et al.*, 2006); the study reported that as the number of symptoms associated with borderline personality disorder increased, so did the levels of psychiatric comorbidity (such as depression, PTSD and GAD), the levels of suicidal and selfharming behaviour, as well as the rates of utilisation of healthcare resources (that is, inpatient stays, outpatient visits and emergency department visits). Moreover, the number of symptoms observed was positively related to rates of incarceration and other contacts with military forensic services (which are expected to incur extra costs). Psychiatric comorbidity is common in people with borderline personality disorder (Bender *et al.*, 2001; Black *et al.*, 2006) and, when present, results in a significant increase in total healthcare costs (Bender *et al.*, 2001; Rendu *et al.*, 2002).

The reported resource use and cost estimates have been made by studying people with borderline personality disorder in contact with health services. However, it is known that a significant proportion of people with personality disorders fail to seek treatment and, when they do, future disengagement with services is quite common. Moreover, contacts with social services, problems with housing, levels of unemployment and involvement with the criminal justice system incur further substantial costs that have not been thoroughly examined, if at all. Therefore, the financial and psychological implications of borderline personality disorder to society are likely to be wider than those suggested in the literature. Efficient use of available healthcare resources is required to maximise the benefits for people with borderline personality disorder, their family and carers, and society in general.

3. METHODS USED TO DEVELOP THIS GUIDELINE

3.1 OVERVIEW

The development of this guideline drew upon methods outlined by NICE (*The Guidelines Manual* ¹ [NICE, 2006b]). A team of healthcare professionals, lay representatives and technical experts known as the Guideline Development Group (GDG), with support from the NCCMH staff, undertook the development of a person-centred, evidence-based guideline. There are six basic steps in the process of developing a guideline:

- define the scope, which sets the parameters of the guideline and provides a focus and steer for the development work
- define clinical questions considered important for practitioners and service users
- develop criteria for evidence searching and search for evidence
- design validated protocols for systematic review and apply to evidence recovered by the search
- synthesise and (meta-) analyse data retrieved, guided by the clinical questions, and produce evidence profiles
- answer clinical questions with evidence-based recommendations for clinical practice.

The clinical practice recommendations made by the GDG are therefore derived from the most up-to-date and robust evidence base for the clinical and cost effectiveness of the treatments and services used in the treatment and management of borderline personality disorder. In addition, to ensure a service user and family/carer focus, the concerns of service users and families/carers regarding health and social care have been highlighted and addressed by recommendations agreed by the whole GDG.

3.2 THE SCOPE

Guideline topics are selected by the Department of Health and the Welsh Assembly Government, which identify the main areas to be covered by the guideline in a specific remit (see *The Guideline Development Process – An Overview for Stakeholders, the Public and the NHS (Second Edition)* [NICE, 2006c]²). The remit for this guideline was translated into a scope document by staff at the NCCMH.

The purpose of the scope was to:

¹ Available from www.nice.org.uk

² Available from: www.nice.org.uk

- provide an overview of what the guideline will include and exclude
- identify the key aspects of care that must be included
- set the boundaries of the development work and provide a clear framework to enable work to stay within the priorities agreed by NICE and the NCCMH and the remit from the Department of Health/Welsh Assembly Government
- inform the development of the clinical questions and search strategy
- inform professionals and the public about the expected content of the guideline
- keep the guideline to a reasonable size to ensure that its development can be carried out within an 18-month period.

The draft scope was subject to consultation with stakeholders over a 4-week period. During the consultation period, the scope was posted on the NICE website (www.nice.org.uk). Comments were invited from stakeholder organisations and the Guideline Review Panel (GRP). Further information about the GRP can also be found on the NICE website (www.nice.org.uk). The NCCMH and NICE reviewed the scope in light of comments received, and the revised scope was signed off by the GRP.

3.3 THE GUIDELINE DEVELOPMENT GROUP

The GDG was made up of professionals in psychiatry, clinical psychology, nursing, and general practice; academic experts in psychiatry and psychology; two former service users and a carer. The guideline development process was supported by staff from the NCCMH, who undertook the clinical and health economics literature searches, reviewed and presented the evidence to the GDG, managed the process, and contributed to drafting the guideline.

3.3.1 Guideline Development Group meetings

Seventeen GDG meetings were held between January 2007 and September 2008. During each day-long GDG meeting, in a plenary session, clinical questions and clinical and economic evidence were reviewed and assessed, and recommendations formulated. At each meeting, all GDG members declared any potential conflicts of interest, and service user and carer concerns were routinely discussed as part of a standing agenda.

3.3.2 Topic groups

The GDG divided its workload along clinically relevant lines to simplify the guideline development process, and GDG members formed smaller topic groups to undertake guideline work in that area of clinical practice. Topic group 1 covered questions relating to pharmacological interventions; topic group 2 covered psychological

Methods used to develop this guideline

therapies (with a sub-group covering therapeutic communities); topic group 3 covered services; topic group 4 covered young people; and topic group 5 covered service user and family/carer issues. These groups were designed to manage the appraisal of the evidence more efficiently before presenting it to the GDG as a whole. Each topic group was chaired by a GDG member with expert knowledge of the topic area (one of the healthcare professionals or service users as appropriate). Topic groups refined the clinical questions, refined the clinical definitions of treatment interventions, reviewed and prepared the evidence with NCCMH staff before presenting it to the GDG as a whole, and also helped the GDG to identify further expertise in the topic. Topic group leaders reported the status of the group's work as part of the standing agenda. They also introduced and led the GDG discussion of the evidence review for that topic and assisted the GDG Chair in drafting the section of the guideline relevant to the work of each topic group.

3.3.3 Service users and families/carers

Individuals with direct experience of services gave an integral service user/carer focus to the GDG and the guideline. The GDG included two former service users and one carer. They contributed as full GDG members to writing the clinical questions, helping to ensure that the evidence addressed their views and preferences, highlighting sensitive issues and terminology relevant to the guideline, and bringing service user/carer research to the attention of the GDG. In drafting the guideline, they contributed to writing a chapter on service user and family/carer issues for the full guideline, and to formulating recommendations from the service user and family/carer perspective.

3.3.4 Special advisors

Special advisors, who had specific expertise in one or more aspects of treatment and management relevant to the guideline, assisted the GDG, commenting on specific aspects of the developing guideline, including attending topic group meetings and teleconferences if appropriate. Appendix 3 lists those who acted as special advisors.

3.3.5 Researchers contacted for unpublished studies

National and international experts in the area under review were identified through the literature search and through the experience of the GDG members. These experts were contacted to recommend unpublished or soon-to-be published studies in order to ensure up-to-date evidence was included in the development of the guideline. They informed the group about completed trials at the pre-publication stage, systematic

reviews in the process of being published, studies relating to the cost effectiveness of treatment and trial data if the GDG could be provided with full access to the complete trial report. Appendix 5 lists researchers who were contacted.

3.3.6 Peer reviewers

Peer reviewers were identified by the GDG to review the guideline during the consultation phase, as well as stakeholders. In addition, the review of pharmacological treatments was sent to international experts for peer review during the guideline development process because this section of the guideline was completed ahead of time and the draft recommendations were potentially controversial because they contradicted current clinical opinion. Therefore peer reviewers who were leaders in the field were appointed; they were named as special advisers to ensure that confidentiality was maintained (see Appendix 3). Their comments and responses from the GDG are presented in Appendix 11.

3.4 CLINICAL QUESTIONS

Clinical questions were used to guide the identification and interrogation of the evidence base relevant to the topic of the guideline. Before the first GDG meeting, draft questions were prepared by NCCMH staff based on the scope. They were then discussed by the GDG at their first two meetings and a final list drawn up. Where appropriate, the questions were refined once the evidence had been searched and, where necessary, subquestions were generated. The final list of clinical questions can be found in Appendix 6.

For questions about interventions, the PICO (patient, intervention, comparison and outcome) framework was used. This structured approach divides each question into four components: the patients (the population under study), the interventions (what is being done), the comparisons (other main treatment options) and the outcomes (the measures of how effective the interventions have been) (see Text box 2).

Text box 2: Features of a well-formulated question on effectiveness intervention – the PICO guide

Patients/population	Which patients or population of patients are we interested in? How can they be best described? Are there subgroups that need to be considered?
Intervention	Which intervention, treatment or approach should be used?
Comparison	What is/are the main alternative/s to compare with the intervention?

Methods used to develop this guideline

Outcome	What is really important for the patient? Which outcomes should be considered: intermediate or short-term measures; mortality; morbidity and treatment complications; rates of relapse; late morbidity and readmission; return to work, physical and social functioning and other measures such as quality of life; general health status; costs?
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To help facilitate the literature review, a note was made of the best study design to answer each question. There are four main types of clinical question of relevance to NICE guidelines. These are listed in Text box 3. For each type of question, the best primary study design varies, where ‘best’ is interpreted as ‘least likely to give misleading answers to the question’. However, in all cases, a well-conducted systematic review of the appropriate type of study is likely to yield a better answer than a single study. Deciding on the best design type to answer a specific clinical or public health question does not mean that studies of different design types addressing the same question were discarded.

Text box 3: Best study design to answer each type of question

Type of question	Best primary study design
Effectiveness or other impact of an intervention	Randomised controlled trial (RCT); other studies that may be considered in the absence of an RCT are the following: internally/externally controlled before and after trial, interrupted time-series
Accuracy of information (for example, risk factor, test, prediction rule)	Comparing the information against a valid gold standard in a randomised trial or inception cohort study
Rates (of disease, patient experience, rare side effects)	Cohort, registry, cross-sectional study
Costs	Naturalistic prospective cost study

3.5 SYSTEMATIC CLINICAL LITERATURE REVIEW

The aim of the clinical literature review was to systematically identify and synthesise relevant evidence from the literature in order to answer the specific clinical questions developed by the GDG. Thus, clinical practice recommendations are evidence-based, where possible. If evidence was not available, informal consensus methods were used (see Section 3.5.9) and the need for future research specified.

3.5.1 Methodology

A stepwise, hierarchical approach was taken to locating and presenting evidence to the GDG. The NCCMH developed this process based on methods set out in *The Guidelines Manual*³ (NICE, 2006b) and after considering recommendations from a range of other sources. These included:

- Clinical Policy and Practice Program of the New South Wales Department of Health (Australia)
- Clinical Evidence online
- The Cochrane Collaboration
- New Zealand Guidelines Group
- NHS Centre for Reviews and Dissemination
- Oxford Centre for Evidence-Based Medicine
- Scottish Intercollegiate Guidelines Network (SIGN)
- United States Agency for Healthcare Research and Quality
- Oxford Systematic Review Development Programme
- Grading of Recommendations: Assessment, Development and Evaluation (GRADE) Working Group.

3.5.2 The review process

After the scope was finalised, a more extensive search for existing systematic reviews and published guidelines was undertaken to inform the review process. The review team, in conjunction with the GDG, assessed the available existing systematic reviews for relevance to the clinical questions. This helped to assess the quantity and likely quality of available primary research. The initial approach taken to locating primary level studies depended on the type of clinical question and availability of evidence.

The GDG then decided which questions were best addressed by good practice based on expert opinion, which questions were likely to have a good evidence base and which questions were likely to have little or no directly relevant evidence. Recommendations based on good practice were developed by informal consensus of the GDG. For questions with a good evidence base, the review process depended on the type of key question. For questions that were unlikely to have a good evidence base, a brief descriptive review was initially undertaken by a member of the GDG.

Searches

The standard mental health related bibliographic databases were searched including EMBASE, MEDLINE, PsycINFO, and Central, together with the grey literature database HMIC. Search filters developed by the review team consisted of a combination of subject heading and free-text phrases. Specific filters were developed

³ Available from www.nice.org.uk

Methods used to develop this guideline

for the guideline topic and, where necessary, for individual clinical questions (see relevant chapters for details). The topic-specific filters were combined with appropriate research design filters developed for systematic reviews, RCTs and other appropriate research designs (Appendix 7).

The review team also scanned the reference lists of included studies and existing systematic reviews for additional references, together with evidence submitted by stakeholders. Unpublished evidence was also sought (see below). In addition, the tables of contents of appropriate journals were checked regularly for relevant studies. Searches for evidence were re-run every 6 months during the guideline development process with the final search undertaken between 6 and 8 weeks before submission of the consultation drafts. After this point, studies were included only if they were judged by the GDG to be exceptional (for example, the evidence was likely to change a recommendation).

The search process for questions concerning interventions

For questions related to interventions, the initial evidence base was formed from wellconducted RCTs that addressed at least one of the clinical questions (the review process is illustrated in Flowchart 1). Although there are a number of difficulties with the use of RCTs in the evaluation of interventions in mental health, the RCT remains the most important method for establishing treatment efficacy (this is discussed in more detail in the appropriate clinical evidence chapters). For other clinical questions, searches were for the appropriate study design (see above).

Since it was known from a review of existing systematic reviews in this area that the evidence base for borderline personality disorder was relatively small, a search for all RCTs for this topic area was undertaken together regardless of intervention.

After the initial search results were scanned liberally to exclude irrelevant papers, the review team used a purpose-built study information database to manage both the included and the excluded studies (eligibility criteria were developed after consultation with the GDG). For questions without good-quality evidence (after the initial search), a decision was made by the GDG about whether to (a) repeat the search using subject-specific databases (for example, CINAHL, AMED, SIGLE or PILOTS), (b) conduct a new search for lower levels of evidence, or (c) adopt a consensus process (see Section 3.5.9).

Study selection

All primary-level studies included after the first scan of citations were acquired in full and re-evaluated for eligibility at the time they were being entered into the study information database. Appendix 8 lists the standard inclusion and exclusion criteria. More specific eligibility criteria were developed for each clinical question and are described in the relevant clinical evidence chapters. Studies were critically appraised for methodological quality (see Appendix 9 and Appendix 16). The eligibility of each study was confirmed by at least one member of the appropriate topic group.

Methods used to develop this guideline

For some clinical questions, it was necessary to prioritise the evidence with respect to the UK context (that is, external validity). To make this process explicit, the topic groups took into account the following factors when assessing the evidence:

- participant factors (for example, comorbid diagnoses and setting)
- provider factors (for example, model fidelity, the conditions under which the intervention was performed and the availability of experienced staff to undertake the procedure)
- cultural factors (for example, differences in standard care and differences in the welfare system).

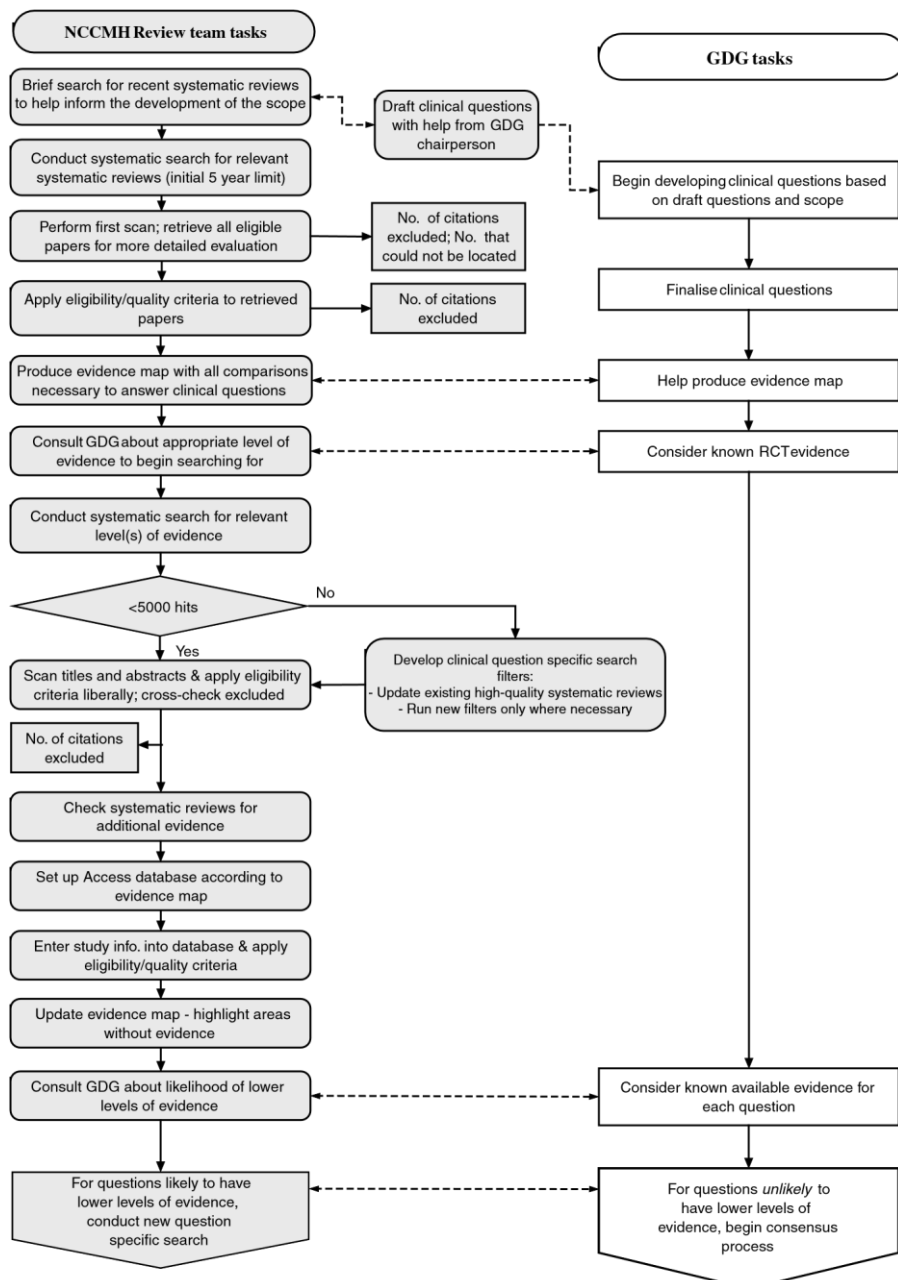
It was the responsibility of each topic group to decide which prioritisation factors were relevant to each clinical question in light of the UK context and then decide how they should modify their recommendations.

Unpublished evidence

The GDG used a number of criteria when deciding whether or not to accept unpublished data. First, the evidence must have been accompanied by a trial report containing sufficient detail to properly assess the quality of the data. Second, the

Flowchart 1: Guideline review process

Methods used to develop this guideline



evidence must have been submitted with the understanding that data from the study and a summary of the study's characteristics would be published in the full guideline. Therefore, the GDG did not accept evidence submitted as commercial in confidence. However, the GDG recognised that unpublished evidence submitted by investigators

might later be retracted by those investigators if the inclusion of such data would jeopardise publication of their research.

3.5.3 Outcomes

Outcome measurement on borderline personality disorder is problematic, partly because of the nature of the disorder and partly because of the relative immaturity of intervention research in this field. Since diagnosis of the disorder is based on the presence of five symptoms out of a possible total of nine with no requirement for the presence of particular symptoms (based on DSM-IV which is used by most treatment studies), trialists usually measure outcomes on all or some of these symptoms. In addition, more than one outcome measure has been developed for most areas of psychopathology caused by the disorder as well as psychosocial functioning affected by the disorder.

In order to deal with the plethora of outcomes reported by the trials forming the guideline's evidence base, the GDG appointed a special advisor with expertise in this area (see Appendix 3). A list of outcomes reported by the studies considered by the GDG is in Appendix 10, together with information on which were used and which were not. For a rating scale to be considered a validation study had to be published in a peer-reviewed journal. In order to increase the power of the meta-analyses, scales reporting the same outcome were examined in detail to assess whether they could be combined. However, self-report and clinical-rated scales were not combined.

3.5.4 Data extraction

Outcome data were extracted from all eligible studies, which met the quality criteria, using a standardised form (see Appendix 8). Study characteristics were also extracted into an Access database. Full study characteristics are in Appendix 16 with summary tables in the evidence chapters.

For a given outcome (continuous and dichotomous), where more than 50% of the number randomised to any group were not accounted for⁴ by trial authors, the data were excluded from the review because of the risk of bias. However, where possible, dichotomous efficacy outcomes were calculated on an intention-to-treat (ITT) basis (that is, a 'once-randomised-always-analyse' basis). This assumes that those participants who ceased to engage in the study—from whatever group—had an unfavourable outcome. This meant that the 50% rule was not applied to dichotomous outcomes where there was good evidence that those participants who ceased to engage in the study were likely to have an unfavourable outcome (in this case, early

⁴ 'Accounted for' in this context means using an appropriate method for dealing with missing data (for example, last observation carried forward or a regression technique).

Methods used to develop this guideline

withdrawals were included in both the numerator and denominator). Adverse effects were entered into Review Manager 4.2.8 (Cochrane Collaboration, 2005) as reported by the study authors because it was usually not possible to determine whether early withdrawals had an unfavourable outcome. For the outcome 'leaving the study early for any reason', the denominator was the number randomised.

Where some of the studies failed to report standard deviations (for a continuous outcome), and where an estimate of the variance could not be computed from other reported data or obtained from the study author, the following approach was taken⁵:

1. When the number of studies with missing standard deviations was small and when the total number of studies was large, the pooled standard deviation from all the other available studies in the same meta-analysis was used. In this case, the appropriateness of the imputation was made by comparing the standardised mean differences (SMDs) of those trials that had reported standard deviations against the hypothetical SMDs of the same trials based on the imputed standard deviations. If they converged, the meta-analytical results were considered to be reliable.
2. When the number of studies with missing standard deviations was large or when the total number of studies was small, standard deviations were taken from a previous systematic review (where available), because the small sample size may allow unexpected deviation due to chance. In this case, the results were considered to be less reliable.

Consultation was used to overcome difficulties with coding. Data from studies included in existing systematic reviews were extracted independently by one reviewer and cross-checked with the existing dataset. Where possible, two independent reviewers extracted data from new studies. Where double data extraction was not possible, data extracted by one reviewer was checked by the second reviewer. Disagreements were resolved with discussion. Where consensus could not be reached, a third reviewer resolved the disagreement. Masked assessment (that is, blind to the journal from which the article comes, the authors, the institution and the magnitude of the effect) was not used since it is unclear that doing so reduces bias (Jadad *et al.*, 1996; Berlin, 1997).

3.5.5 Synthesising the evidence

Where possible, meta-analysis was used to synthesise the evidence using Review Manager 4.2.8 (Cochrane Collaboration, 2005). If necessary, reanalyses of the data or sub-analyses were used to answer clinical questions not addressed in the original studies or reviews.

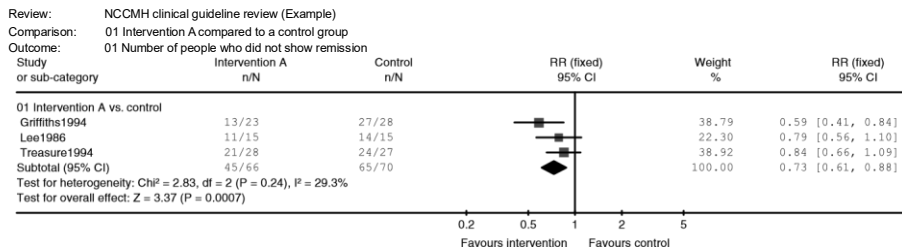
Dichotomous outcomes were analysed as relative risks (RR) with the associated 95% confidence interval (CI) (for an example, see Figure 1). A relative risk (also called

⁵ Based on the approach suggested by Furukawa and colleagues (2006).

a risk ratio) is the ratio of the treatment event rate to the control event rate. An RR of 1 indicates no difference between treatment and control. In Figure 1, the overall RR of 0.73 indicates that the event rate (that is, non-remission rate) associated with intervention A is about three quarters of that with the control intervention or, in other words, the relative risk reduction is 27%.

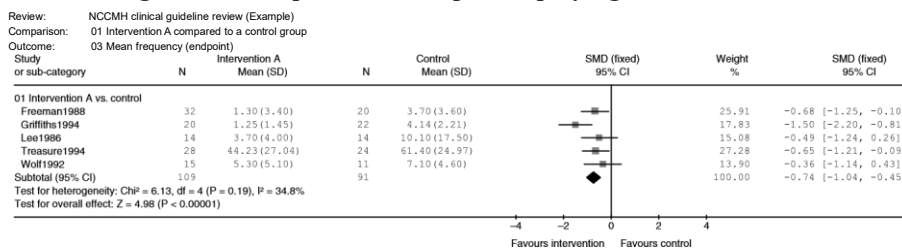
The CI shows with 95% certainty the range within which the true treatment effect should lie and can be used to determine statistical significance. If the CI does not cross the 'line of no effect', the effect is statistically significant at the 5% significance level.

Figure 1: Example of a forest plot displaying dichotomous data



Continuous outcomes were analysed as weighted mean differences (WMD), or as a standardised mean difference (SMD) when different measures were used in different studies to estimate the same underlying effect (for an example, see Figure 2). If provided, ITT data, using a method such as 'last observation carried forward', were preferred over data from completers.

Figure 2: Example of a forest plot displaying continuous data



To check for consistency between studies, both the I² test of heterogeneity and a visual inspection of the forest plots were used. The I² statistic describes the proportion of total variation in study estimates that is due to heterogeneity (Higgins & Thompson, 2002). The I² statistic was interpreted in the following way:

- 50%: notable heterogeneity (an attempt was made to explain the variation, for example outliers were removed from the analysis or sub-analyses were conducted to examine the possibility of moderators. If studies with heterogeneous results were found to be comparable, a random-effects model was used to summarise the results [DerSimonian & Laird, 1986]. In the random-effects analysis,

Methods used to develop this guideline

heterogeneity is accounted for both in the width of CIs and in the estimate of the treatment effect. With decreasing heterogeneity the random-effects approach moves asymptotically towards a fixed-effects model).

- 30 to 50%: moderate heterogeneity (both the chi-squared test of heterogeneity and a visual inspection of the forest plot were used to decide between a fixed and random-effects model)
- 30%: mild heterogeneity (a fixed-effects model was used to synthesise the results).

To explore the possibility that the results entered into each meta-analysis suffered from publication bias, data from included studies were entered, where there were sufficient data, into a funnel plot. Asymmetry of the plot was taken to indicate possible publication bias and investigated further.

An estimate of the proportion of eligible data that were missing (because some studies did not include all relevant outcomes) was calculated for each analysis.

The number needed to treat–benefit (NNTB) or the number needed to treat–harm (NNTH) was reported for each outcome where the baseline risk (that is, control group event rate) was similar across studies. In addition, NNTs calculated at follow-up were only reported where the length of follow-up was similar across studies. When the length of follow-up or baseline risk varies (especially with low risk), the NNT is a poor summary of the treatment effect (Deeks, 2002). The percentage with the event in question was reported for each treatment group.

Included/excluded studies tables, generated automatically from the study information database, were used to summarise general information about each study (see Appendix 16). Where meta-analysis was not appropriate and/or possible, the reported results from each primary-level study were also presented in the included studies table (and included, where appropriate, in a narrative review).

Skewed data

Continuous data reported by the trials may not be normally distributed. While this is not so much of a problem in larger trials, effect sizes calculated from data from smaller trials should be treated with caution. Given that many of the trials reviewed for this guideline used relatively small populations, skewedness was assessed based on the definition that the mean is greater than two times the standard deviation. Evidence was downgraded where skewed data existed (see section on evidence profile tables below). All effect sizes calculated with skewed data are marked with an asterisk and should therefore be interpreted cautiously.

3.5.6 Presenting the data to the GDG

Summary characteristics tables and, where appropriate, forest plots generated with Review Manager 4.2.8 (Cochrane Collaboration, 2005) were presented to the GDG in order to prepare an evidence profile for each review and to develop recommendations.

Evidence profile tables

An evidence profile table was used to summarise both the quality of the evidence and the results of the evidence synthesis (see Table 3 for an example of an evidence profile table). Each table included details about the quality assessment of each outcome: quality of the included studies based on the SIGN grade (see Appendix 9 for checklist), number of studies, and limitations, information about the consistency of the evidence (see below for how consistency was measured), directness of the evidence (that is, how closely the outcome measures, interventions and participants match those of interest) and any other considerations (for example, effect sizes with wide CIs would be described as imprecise data). Each evidence profile also included a summary of the findings: number of patients included in each group, an estimate of the magnitude of the effect, quality of the evidence, and the importance of the evidence. The quality of the evidence was based on the quality assessment components (study design, limitations to study quality, consistency, directness and any other considerations) and graded using the following definitions:

- **High** Further research is very unlikely to change confidence in the estimate of the effect
- **Moderate** Further research is likely to have an important impact on confidence in the estimate of the effect and may change the estimate
- **Low** Further research is very likely to have an important impact on confidence in the estimate of the effect and is likely to change the estimate
- **Very low** Any estimate of effect is very uncertain.

For further information about the process and the rationale of producing an evidence profile table, see GRADE Working Group (2004). Full evidence profiles are in Appendix 18 and summary profiles are included in the evidence chapters.

Forest plots

Each forest plot displayed the effect size and CI for each study as well as the overall summary statistic. The graphs were organised so that the display of data in the area to the left of the 'line of no effect' indicated a 'favourable' outcome for the treatment in question. Forest plots are in Appendix 17.

3.5.7 Determining clinical significance

In order to facilitate consistency in generating and drafting the clinical summaries, a decision tree was used to help determine, for each comparison, the likelihood of the effect being clinically significant (see Figure 3). The decision tree was designed to be used as one step in the interpretation of the evidence (primarily to separate clinically important from clinically negligible effects) and was not designed to replace clinical judgement. For each comparison, the GDG defined *a priori* a clinically significant threshold, taking into account both the comparison group and the outcome.

Methods used to develop this guideline

As shown in Figure 3, the review team first classified the point estimate of the effect as clinically significant or not. For example, if an RR of 0.75 was considered

Table 3: Example evidence profile for brief psychological interventions

Forest plot (all 100% factors)	Description of study population (100% factors)	Study quality (SMD)	Consistency (I ²)	Directness (Other N)	Effect size (BPD)	Absolute Likelihood of control	Overall WMD (95% CI)	Important effect
Anxiety Measures								
Psychopharmacology (MACT vs TAU)	MACT vs TAU	MACT vs TAU	SIGN 1	N/A	Sparse, 31/33 SMD	data (1)	WMD (0.5) (0.01, 2.41)	Unlikely/Moderate
Depression measures								
Psychopharmacology (MACT vs TAU)	MACT vs TAU	MACT vs TAU	SIGN 1	N/A	Sparse, 31/33 SMD	skewed and inconclusive data (2)	WMD (0.42, 0.07) (0.07, 4.42)	Inconclusive/Very low
Psychopharmacology (MACT vs TAU)	MACT vs TAU	MACT vs TAU	SIGN 1	N/A	Sparse, 31/33 SMD	skewed and inconclusive data (2)	WMD (0.37, 0.12) (0.12, 2.12)	Inconclusive/Very low

Continued

03.01

03.02

03.02

Table 3:(Continued)

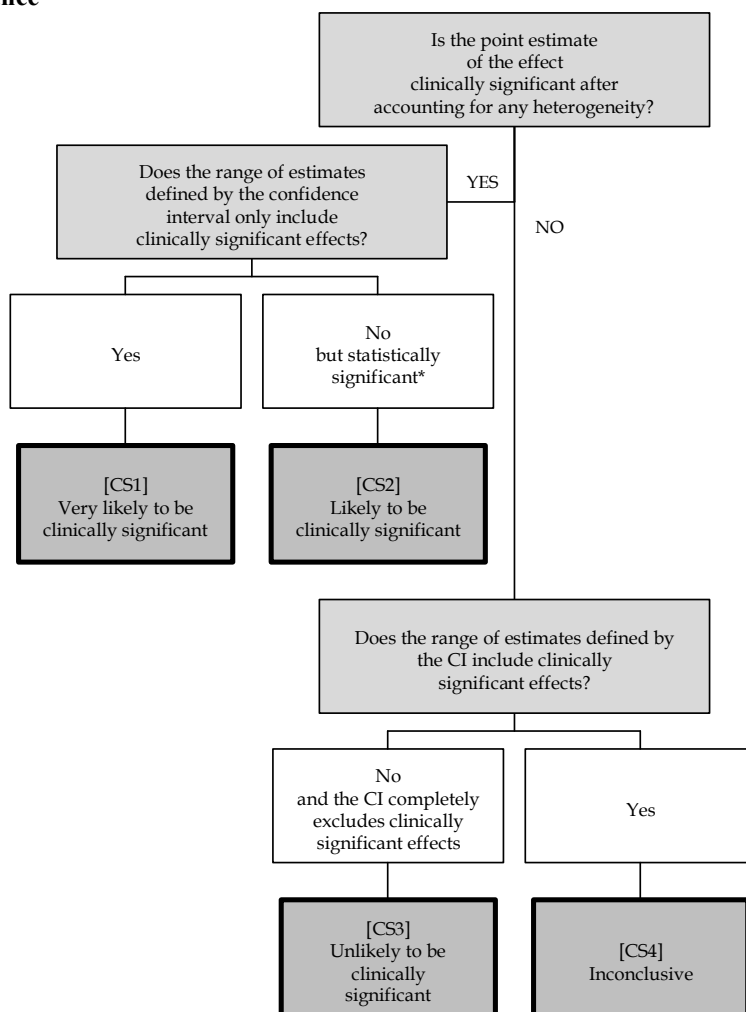
control		) WMD	important effect	
Self-harm and suicidal acts (reported together) (continuous)				
	1.67, 88.0.1)(	5.68 , 0.38)(	favouring treatment)	
skewed and 15/15(	1.24, 0.22)(0.51	1.74) , 11.16 71		
Self-harm measures (dichotomous)				
	1.07) , 88			

Forest DescriptionStudy IdsQuality ConsistencyDirectness Other N treatment Effect size Absolute Likelihood of Overall plot(all 100% factorsgroup/N (SMD)statistic clinically quality	
N/ASparse and 15/13SMD	1)
N/ASparse, SMD	inconclusive (2)
N/ASparse 34/36RR	data (1)

		BPD)
acts reported data (and suicidal 2006 MACIs skewed together data		

	Psych 01 Self-harm Weinberg SIGN 1	
	together	
	suicidal 2006 MACT	Psych 02 Self-harm Weinberg SIGN 1
	acts reported	
	month	
	follow-up)	
03.05	MACT episode of parasuicide	Psych 01 No. with Tyrer 2003 SIGN 1

Figure 3: Decision tree for helping to judge the likelihood of clinical significance



*Efficacy outcomes with large effect sizes and very wide confidence intervals should be interpreted with caution and should be described as inconclusive (CS4), especially if there is only one small study.

to be the threshold, then a point estimate of 0.73 (as can be seen in Figure 1), would meet the criteria for clinical significance. Where heterogeneity between studies was judged problematic, in the first instance an attempt was made to explain the cause of the heterogeneity (for example, outliers were removed from the analysis or sub-analyses were conducted to examine the possibility of moderators). Where homogeneity could not be achieved, a random-effects model was used.

Methods used to develop this guideline

Where the point estimate of the effect exceeded the threshold, a further consideration was made about the precision of the evidence by examining the range of estimates defined by the CI. Where the effect size was judged clinically significant for the full range of plausible estimates, the result was described as *very likely to be clinically significant* (that is, CS1). In situations where the CI included clinically unimportant values, but the point estimate was both clinically and statistically significant, the result was described as *likely to be clinically significant* (CS2). However, if the CI crossed the line of no effect (that is, the result was not statistically significant), the result was described as *inconclusive* (CS4).

Where the point estimate did not meet the criteria for clinical significance and the CI completely excluded clinically significant values, the result was described as *unlikely to be clinically significant* (CS3). Alternatively, if the CI included both clinically significant and clinically unimportant values, the result was described as *inconclusive* (CS4). In all cases described as inconclusive, the GDG used clinical judgement to interpret the results.

3.5.8 Forming the clinical summaries and recommendations

Once the evidence profile tables relating to a particular clinical question were completed, summary tables incorporating important information from the evidence profile and an assessment of the clinical significance of the evidence were produced (these tables are presented in the evidence chapters). Finally, the systematic reviewer along with the topic group lead produced a clinical summary. Once the evidence profile tables and clinical summaries were finalised and agreed by the GDG, the associated recommendations were produced, taking into account the trade-off between the benefits and risks as well as other important factors. These included economic considerations, values of the development group and society, and the GDG's awareness of practical issues (Eccles *et al.*, 1998).

3.5.9 Method used to answer a clinical question in the absence of appropriately designed, high-quality research

In the absence of level I evidence (or a level that is appropriate to the question), or where the GDG were of the opinion (on the basis of previous searches or their knowledge of the literature) that there were unlikely to be such evidence, either an informal or formal consensus process was adopted. This process focused on those questions that the GDG considered a priority.

Informal consensus

The starting point for the process of informal consensus was that a member of the topic group identified, with help from the systematic reviewer, a narrative review that

most directly addressed the clinical question. Where this was not possible, a brief review of the recent literature was initiated.

This existing narrative review or new review was used as a basis for beginning an iterative process to identify lower levels of evidence relevant to the clinical question and to lead to written statements for the guideline. The process involved a number of steps:

1. A description of what is known about the issues concerning the clinical question was written by one of the topic group members.
2. Evidence from the existing review or new review was then presented in narrative form to the GDG and further comments were sought about the evidence and its perceived relevance to the clinical question.
3. Based on the feedback from the GDG, additional information was sought and added to the information collected. This may include studies that did not directly address the clinical question but were thought to contain relevant data.
4. If, during the course of preparing the report, a significant body of primary-level studies (of appropriate design to answer the question) were identified, a full systematic review was undertaken.
5. At this time, subject possibly to further reviews of the evidence, a series of statements that directly addressed the clinical question were developed.
6. Following this, on occasions and as deemed appropriate by the development group, the report was then sent to appointed experts outside the GDG for peer review and comment. The information from this process was then fed back to the GDG for further discussion of the statements.
7. Recommendations were then developed.
8. After this final stage of comment, the statements and recommendations were again reviewed and agreed upon by the GDG.

3.6 HEALTH ECONOMICS REVIEW STRATEGIES

The aim of the health economics was to contribute to the guideline's development by providing evidence on the cost effectiveness of interventions for people with borderline personality disorder covered in the guideline. For this reason, a systematic literature review of existing economic evidence in this area was conducted.

3.6.1 Search strategy

For the systematic review of economic evidence the standard mental-health-related bibliographic databases (EMBASE, MEDLINE, CINAHL and PsycINFO) were searched. For these databases, a health economics search filter adapted from the Centre for Reviews and Dissemination at the University of York was used in combination with the general strategy for borderline personality disorder. Additional

Methods used to develop this guideline

searches were performed in specific health economics databases (NHS EED, OHE HEED), as well as in the HTA database. For the HTA and NHS EED databases, the general strategy for borderline personality disorder was used. OHE HEED was searched using a shorter, database-specific strategy. Initial searches were performed in January 2007. The searches were updated regularly, with the final search performed in May 2008. Details on the search strategies adopted for the systematic review of economic evidence are provided in Appendix 12.

In parallel to searches of electronic databases, reference lists of eligible studies and relevant reviews were searched by hand. Studies included in the clinical evidence review were also screened for economic evidence.

The systematic search of the literature resulted in 3,656 references in total. Publications that were clearly not relevant to the topic (that is, did not provide any information on the economics of borderline personality disorder) were excluded first. The abstracts of all potentially relevant publications (58 papers) were then assessed against a set of inclusion criteria by the health economist. Full texts of the studies potentially meeting the inclusion criteria (including those for which eligibility was not clear from the abstract) were obtained. At this stage, 30 studies had been selected. Studies that did not meet the inclusion criteria, were duplicates, were secondary publications of one study, or had been updated in more recent publications were subsequently excluded. Finally, 18 studies that provided information on the economics of borderline disorder were selected. Of these, ten were cost-of-illness studies or studies that reported data on healthcare resource use associated with borderline personality disorder in general, and eight were economic evaluations of specific interventions for people with borderline personality disorder covered in this guideline. All economic evaluations eligible for inclusion in the systematic review of economic literature were critically appraised according to the checklists used by the *British Medical Journal* to assist referees in appraising full and partial economic analyses (Drummond & Jefferson, 1996) (Appendix 13).

3.6.2 Inclusion/exclusion criteria

The following inclusion/exclusion criteria were applied to select studies identified by the economic searches for further analysis:

- No restriction was placed on language or publication status of the papers.
- Studies published from 1996 onwards were included. This date restriction was imposed in order to obtain data relevant to current healthcare settings and costs.
- Only studies from Organisation for Economic Co-operation and Development countries were included, as the aim of the review was to identify economic information transferable to the UK context.
- Selection criteria based on types of clinical conditions and patients were identical to the clinical literature review; the intention was to include studies that provided data exclusively on people with borderline personality disorder; however, when

no studies answering a specific economic question met this criterion, the criterion was relaxed and economic studies considering a wider study population relevant to people with borderline personality disorder were included in the review, following consensus of the GDG.

- Studies were included provided that sufficient details regarding methods and results were available to enable the methodological quality of the study to be assessed, and provided that the study's data and results were extractable; poster presentations or abstracts were excluded from the review.
- Full economic evaluations that compared two or more relevant options and considered both costs and consequences (that is, cost–consequence analyses, costeffectiveness analyses, cost–utility analyses or cost–benefit analyses) as well as partial economic evaluations (that is, costing analyses) were included in the systematic review.
- Economic studies that omitted intervention costs from the analysis were excluded from the review because their results were considered potentially misleading.

3.6.3 Data extraction

Data were extracted by the health economist using a standard economic data extraction form (Appendix 14).

3.6.4 Presentation of economic evidence

The economic evidence identified by the health economics systematic review is summarised in the respective chapters of the guideline, following presentation of the clinical evidence. The characteristics and results of all economic studies included in the review are provided in the form of evidence tables in Appendix 15.

3.6.5 Economic modelling

Formal decision-analytic economic modelling was not undertaken, owing to lack of appropriate data. Overall, availability of clinical data was limited; clinical studies examined different study populations and reported a large number of outcomes, mainly expressed as scores in rating scales, which could not be pooled together and subsequently converted into a meaningful outcome for economic analysis, for example quality adjusted life years (QALYs). In addition, a well-defined treatment pathway that would form the basis for the structure of an economic model does not exist in the area of borderline personality disorder. A recent Health Technology Assessment (HTA) (Brazier *et al.*, 2006) identified the same problems in attempting to undertake formal economic modelling; instead, the authors used an alternative

Methods used to develop this guideline

approach and carried out separate economic analyses for each of the RCTs reviewed in their report. These analyses by Brazier and colleagues (2006) are described in the respective sections of this guideline (in Chapter 5) and have been considered by the GDG when formulating recommendations. However, they are characterised by strong limitations, as acknowledged by the authors of the report (details on the methods adopted by Brazier and colleagues [2006] are provided in Chapter 5). The GDG estimated that adopting the same approach for the additional RCTs included in this guideline that were not covered by Brazier and colleagues (2006) would suffer from the same strong limitations and therefore would not add substantial information that would be useful in decision making. For this reason, no extra economic modelling was undertaken for this guideline and economic considerations were based exclusively on previously published economic evidence.

3.7 STAKEHOLDER CONTRIBUTIONS

Professionals, service users, and companies have contributed to and commented on the guideline at key stages in its development. Stakeholders for this guideline include:

- service user/carer stakeholders: the national service user and carer organisations that represent people whose care is described in this guideline
- professional stakeholders: the national organisations that represent healthcare professionals who are providing services to service users
- commercial stakeholders: the companies that manufacture medicines used in the treatment of borderline personality disorder
- Primary Care Trusts
- Department of Health and Welsh Assembly Government.

Stakeholders have been involved in the guideline's development at the following points:

- commenting on the initial scope of the guideline and attending a briefing meeting held by NICE
- contributing possible clinical questions and lists of evidence to the GDG
- commenting on the draft guideline (see below).

3.8 VALIDATION OF THIS GUIDELINE

Registered stakeholders commented on the draft guideline, which was posted on the NICE website during the 8-week consultation period. The GRP also reviewed the guideline and checked that stakeholders' comments had been addressed.

Following the consultation period, the GDG finalised the recommendations and the NCCMH produced the final documents. These were then submitted to NICE.

Methods used to develop this guideline

NICE then formally approved the guideline and issued its guidance to the NHS in England, Wales and Northern Ireland.

4. EXPERIENCE OF CARE

4.1 INTRODUCTION

This chapter provides an overview of the experience of people with borderline personality disorder and their families/carers. In the first section are first-hand personal accounts written by service users, former service users and a carer, which provide some experiences of having the diagnosis, accessing services and caring for someone with the disorder. It should be noted that these accounts are not representative of the experiences of people with borderline personality disorder, and therefore can only ever be illustrative. The accounts were obtained through contacts of the service users and carers on the GDG, and therefore illustrate a relatively narrow range of experience (the majority are from people whose primary mode of treatment was in a therapeutic community). It should also be borne in mind that writing about borderline personality disorder can be an extremely painful process for many people, which further restricts the number of available personal accounts.

This is followed by a review of the qualitative literature of service user experience and a narrative review of the available evidence and expert consensus regarding families/carers of people with borderline personality disorder. Finally there is a summary of the themes emerging from the personal accounts and the literature reviews, which provides a basis for the recommendations.

4.2 PERSONAL ACCOUNTS

4.2.1 Introduction

This section contains first-hand personal accounts from people with borderline personality disorder and a carer. The accounts offer different perspectives of the disorder: accounts A and B are written by former service users (both female); accounts C (male) and D (female) are written by current service users; and account E is from the mother of the author of account C. The writers of the accounts were contacted through the service user contacts on the GDG; they were asked to write about their experiences of diagnosis, accessing services and treatment, their relationship with healthcare professionals, and self-help and support during a crisis. Each author signed a consent form allowing the account to be reproduced in this guideline.

4.2.2 Personal account A

I'd been a troubled kid from about the age of 9. My Dad worked away a lot and I had a difficult relationship with my Mum; we clashed and there was limited physical affection between us as I got older. In general though, I would say that I had a spoilt,

middleclass upbringing with no material hardships. Despite this I was still unable to cope with the out-of-control emotions inside of me. Looking back I am able to describe these emotions as anger, but at the time I didn't know what they were and they terrified me. I was hurt and lonely but didn't have the words to express how I felt or what I needed.

I remember the first time I started cutting myself. I was sitting in the school field at break time and rubbing a piece of glass up and down my arm. It hurt but the pain felt comforting and it focused my emotions on that point of my skin. When I bled it felt like all the bad feelings just flowed out of me.

From then on, it was as if I had found my escape mechanism. I never had to deal with out-of-control panic, fear, anger, rage or vulnerability again. I could just bleed. By my late teens I was an empty shell. I felt nothing any more, and no one could reach me or hurt me. I lived in a strange, safe, isolated world.

In my isolated world all communication shut down. At home I could count the number of words passing between my Mum and me each day on the fingers of one hand. At school I had friends and was academically successful but people were suspicious about the number of injuries I was developing.

One of my friends had read an article about self-harm and questioned me about it. Even though I was the one putting the razor blade against my arm, I was unable to accept that people would actually cut themselves on purpose and denied it. Teachers became involved but I think my horror at the suggestion of self-harm encouraged almost everyone to believe I was just clumsy.

From school I went on to medical school to train as a doctor. University is a challenging place for someone who struggles with emotions and relationships, and my cutting and other self-injurious behaviour increased quite dramatically in order for me to continue, but I did continue and was getting by. When I first started university I felt as though I had to re-learn how to talk to people – I had shut down so much that I didn't think I could communicate on a social level.

In my second year at university, I was attacked and raped on the way home from a student party. Life started to spiral out of control for me at this point. The bigger my inner turmoil the stronger the need was to bleed. I started making deeper and deeper cuts, sometimes I would go through arteries and need to be hospitalised. I could no longer be described as getting by.

After one such incident I was visited in hospital by a psychiatrist and taken by taxi to the local psychiatric clinic. This was a serious shock to the system – I felt I was descending into an unknown and terrifying world of 'loonies' and 'nutters', and someone thought I was one of them.

I was immediately prescribed chlorpromazine along with assorted antidepressants and the side effects left me feeling at home in the asylum very quickly. My legs were twitchy and my whole body felt lethargic. I wandered around dragging my feet with my head hung low, and soon relaxed into day room behaviour of cigarette smoking, rocking and leg twitching. The drugs had the effect of numbing both my mind and my body and I was able to get through my days without feeling desperately self-

Experience of care

destructive. It was not a good way to be seen by friends though, and I don't think my partner and flatmates ever really got over seeing me like that.

I was diagnosed with post-traumatic stress disorder and depression. I started a course of psychotherapy at the same time, stayed at the clinic for a few weeks and then went back home. I continued with the therapy and my clinical studies but the two didn't combine very well. Psychotherapy can leave you very raw as you deal with any number of complex issues from the past. As I've said before, I didn't deal well with emotion; it was as if I hadn't been taught how to recognise it or deal with it.

I had a very good relationship with my psychotherapist; I trusted her and felt we were getting somewhere, but the trouble with psychotherapy is that you often feel a lot worse before you start to feel better. I had been seeing her for some months when she announced that she was going to have to hand me over to another therapist as she had to move away. I think this came at quite a tough point in the therapy and it coincided with an escalation in my self-harming.

I was spiralling out of control, becoming hugely self-destructive and suicidal and I was quickly readmitted to hospital. I spent a number of days on constant observation, with a nurse staying with me every second of the day, but I still managed to harm myself. It was getting to the point where members of staff were actually putting themselves at risk in order to prevent me from destroying myself.

At this point my psychotherapist called my parents and told them that she didn't believe I would still be alive to see my birthday at the end of the month. I didn't see my parents very often but they had visited me once at the clinic for a family session with my psychotherapist. I can't imagine how they handled this news. Even now that all this is behind us and we enjoy a good bond, I still feel desperately guilty for putting them through that entire trauma.

Shortly before my 22nd birthday I was called into a room with my psychotherapist and GP. They sectioned me and I was taken away to a regional secure unit 'for my own safety'. A secure unit is effectively a medical prison for the criminally mentally ill; it is no place for a distressed, depressed and self-destructive individual. I cannot really complain that my psychotherapist sent me there though; I think in part she was desperately trying to ensure my safety – she felt a certain amount of responsibility as she had to move on, and there really weren't any suitable alternatives at the time.

At the secure unit I found myself on a mixed ward with rapists and arsonists and for the second time I felt out of place. Despite the rigorous searches and removal of all my belongings, I still managed to secure razor blades. As a result, I was strip searched and I spent the next few days sleeping naked on a bomb-proof mattress on the floor of a padded cell, while under permanent observation.

It was here in the secure unit that the forensic psychiatrist gave me the label of 'borderline personality disorder'. Given the nature of my surroundings I felt that I was being punished – I was locked up with people who had committed crimes and my core being, my personality, was under attack.

This particular crisis period was time limited (my panic was related to my birthday), so when that day finally passed safely I began to take control of myself again. I could get through the day without focusing entirely on ways of disposing of

myself and instead I began to look for ways to get out. Thankfully I didn't stay at the secure unit for very long. I had already started to appeal for my section to be quashed, but the staff also felt I was not in the best place – they felt somewhat compromised in retaining me as the only patient on a mental health section rather than one imposed via the courts. I was visited by the consultant at the local therapeutic community and invited for a community assessment.

I wasn't sure how to take this latest development. A therapeutic community had been mentioned to me before and I thought this would involve groups of people having crisis meetings to discuss how it made them 'feel' when someone took their milk from the fridge, for example. Again, I didn't think this was part of my life. I was a medical student – successful academically – but there was no getting around the fact I wasn't coping with living very well. It was unlikely that I'd ever be able to go back to my studies, so I had lost my career, my home and my friends. Life had pretty much reached rock bottom for me so it was time for me to accept any lifeline I was being thrown.

I went to the community meeting and it was clear that this group of about 25 residents were split on whether they wanted me to join them. Half felt I should be given a chance and the other half were adamant that I would be bad for the group. I was considered a big risk given my history of uncontrolled self-harm. Finally they came down on my side and let me join them but on the condition that I'd be out if I cut again.

The therapeutic community was the strangest, toughest, most homely place in which I have ever lived. I was there for about 15 months, learning how to feel and live again. It was as if I was given a second chance to do my growing up.

It's an incredibly challenging environment: if you mess up it affects other people and they don't hold back from telling you. That is really tough. You can be struggling and want to cut, but you have someone facing you in a group telling you how selfish you are and how that would make them feel. It's the group dynamic that gets you through in the end though. I learnt so much from the staff and residents in those 15 months and truly thank them for giving me back my life.

Part of the responsibility of living in a therapeutic community is to take on roles related to the running of the community. This varied between preparing meals, chairing meetings, writing notes on individual group sessions and feeding back after someone has spoken of their individual struggles. I often found myself assuming or being pushed into the role of spokesperson or advocate and the effect was to renew my feelings of self worth.

I arrived unsure of who I was and where I belonged but slowly, through the interaction with others, I was able to reassemble my understanding of me.

When I left the therapeutic community I was in a position to start putting my life back together. It took a while as I'd pretty much reached the bottom rung, but life is good for me now. I'm almost 15 years on and haven't purposefully injured myself in that time. I've had a number of jobs, got myself a career, a PhD and some good friends. It's taken me a long while to pick up from where I left off at 9 years old but I think I'm there now, happy, settled and coping again.

4.2.3 Personal account B

My psychiatrist gave me the diagnosis of borderline personality disorder when I was 24. I was an inpatient in a psychiatric hospital at the time. I had been expecting this for some time, having been aware of borderline personality disorder from my previous work as a nursing assistant in child and adolescent mental health. However, I had been struggling a long time before I realised the diagnosis was applicable to me. Consequently, receiving the diagnosis wasn't a shock, and at that moment I found it reassuring that I wasn't going to tip into a deep psychosis from which I would never return. It also helped me start to piece together my understanding of how I had got to that point – why things had got so bad that the only place I could have any kind of existence was a psychiatric hospital.

Looking back, my whole life had seemed to be heading to that point. As a child I was hyperactive and was more interested in my environment and learning new things than being held by my parents. I think my parents interpreted this as a rejection and as being difficult. In addition, the family dynamics were difficult and incomprehensible to me as a child and I blamed myself for them. However, I lived well and was lucky enough to be able to do most things that I wanted in terms of activities; my parents gave me everything that they could. Despite this, home felt too unsafe and volatile an environment to express my emotional and personal needs. Among my sisters I felt the odd one out. I felt that I didn't belong in my family. My way of coping with these feelings was to throw myself into school, where my joy of learning, music and sport allowed me to immerse myself to the extent that my success at school somehow became a substitute for parenting.

What I didn't realise at the time, however, was that I still had a huge yearning to be parented. I needed emotional connection, safety and understanding but didn't recognise those needs nor knew how to get them met. As I grew older, I struggled more and more socially because what I was missing meant that I did not acquire the empathic understanding needed to manage social relationships. This yearning for connection led me to seek refuge in any potential parenting figures that I came across. Unfortunately, one person who took me under his wing was interested in me for the wrong reasons – I was sexually abused and raped as a child over a period of 6 months to a year. This amplified my difficulties. I became even more socially isolated and emotionally inept as I tried to shut out these experiences that I couldn't begin to comprehend.

Not long after this I moved with my family to a different part of the country. At first, this was a welcome change and a relief from abuse. People had no prior knowledge or judgements about me and this was welcome. I could be different from before – I could start again. However, this relief only lasted for about 6 months. Now as a teenager, my difficulties and the emotions and memories I had temporarily locked away began to resurface. My behaviour at school deteriorated, my moods became unstable, I was withdrawn, I frequently sought out teachers for support but didn't know why, I'd leave lessons for no reason, I'd have arguments with teachers, I began

to self-harm (hitting myself mainly), and became more preoccupied with the thought of suicide.

When I was 14, I was referred to child and adolescent mental health outpatient services where I began work with a clinical psychologist whom I saw weekly, sometimes twice weekly, for approximately 4 to 5 years. I was diagnosed with posttraumatic stress disorder.

Having a psychologist meant that I finally had someone who could partly meet my need for a parent (in that they could give me an emotional connection and understanding I so desperately needed). I undertook some important work around understanding the abuse, but when she tried to initiate conversations about my family I couldn't say anything. All I knew then was that I didn't feel safe at home. She described my family life as being a 'ghost town'.

During this time my thoughts of self-harm and suicide became more prominent, but my drive towards destructiveness was most apparent in my relationships with men. Not knowing how to deal with men after the abuse, the conflict between needing to be close to someone and being frightened of intimacy became increasingly more difficult to handle as I was now at that age where male attention was inevitable. I would find myself in difficult situations where I would end up having sex with people I didn't want to as a result of fear and an inability to express my needs and say 'no'. After a while, I figured the only way to deal with this was to be the one in control. Instead of waiting to be seduced I became the seducer, placing myself in a number of risky situations.

Despite all this, I managed to get to university. Although I thrived in the freedom that university allowed and in being away from my family, I was still extremely fragile in my sense of self and in my emotions. There was still a lot I had to deal with and understand about my past, and this at times, especially combined with the pressure at university, meant that I found it extremely difficult to cope. I accessed the university counselling service on a number of occasions, but found that it didn't work at quite the depth I needed. In the holidays, I occasionally had the opportunity to have a number of sessions with my previous clinical psychologist. This support often enabled me to be 'topped up' just enough in order to survive another term. However, the final year of my degree saw things start to disintegrate; the added pressure combined with my limited resources meant that I had nothing left at times. My closest friendships broke down and I ended up taking two overdoses as I couldn't manage the situation with my friends, the exams, and the thought of leaving university – I wasn't ready to be an adult.

Just prior to these overdoses I had been referred to a psychologist at university and had been prescribed an antidepressant (paroxetine) by my GP. I struggled to work with this psychologist as he took more of a behaviourist approach, which I didn't find at all helpful. I also struggled with beginning a new therapeutic relationship after having had such a positive therapeutic experience with my previous psychologist. I eventually took myself off the antidepressant because I didn't feel that it was helping. Somehow, I managed to complete my degree and returned to live with my parents. As a result of my overdoses at university, my GP wished to refer me to adult mental health services

Experience of care

when I re-registered. This resulted in my referral to another clinical psychologist who I met approximately biweekly. Things had settled since returning from university, but my difficulties hadn't gone away – they were just more in the background. I still struggled a lot of the time, but I was able to keep this more private. I began working as a classroom assistant in a school with children with special needs, which I thoroughly enjoyed. I then started work in child and adolescent mental health. This proved to be a mixed blessing.

Therapeutically, the clinical psychologist and I had just started to unravel some of my family dynamics and make sense of my experiences growing up. I began to understand that my Mum and I had both struggled with insecure attachments throughout our lives and this helped me to understand some of the dysfunctional interactions I so often repeated in my other relationships. The combination of attachment and psychodynamic understanding worked well for me. It captured so much of the unexplained and helped me construct my life story, putting more solid foundations in place for a sense of self to develop. Understanding my Mum's difficulties and, in addition, my Dad's background (his Mum died when he was a teenager and he had had repeated episodes of depression and anxiety) also helped me understand the volatile interactions that often occurred in our family and my parents' capacity to be mildly physically and emotionally abusive at times.

However, doing this type of therapeutic work while working in child and adolescent mental health proved to be a destructive combination. I thoroughly enjoyed the work and felt that I was good at it. However, I was giving so much to the children I was working with and at the same time was more open to my emotions as a result of the therapeutic work I was undertaking. Everyday, I saw in the children how I was feeling inside being acted out in front of me. This triggered so much that when I went home in the evening I couldn't begin to recognise, name or understand the emotions I was feeling. Instead, all I experienced was a huge vacuum. I was being sucked into something I didn't feel I could survive. I literally felt that this feeling would kill me – it was so huge and consuming.

The only way I could handle these feelings and to feel any sense of control was through self-destruction, although more realistically I felt simultaneously out of control and in control at the same time. The drive to self-destruct was so strong that I felt I had no choice but to self-harm; but through the act I also found some way of regaining some temporary stability, relief from that vacuum, and some control. Previously, I had kept busy to keep this emotion at bay, but as time went on and the therapeutic work continued I couldn't do enough to stop feeling the emotions: overdosing, cutting, burning, blood-letting, balancing precariously on the top of car parks and bridges hoping I could throw myself off them – I tried almost everything. By day I was going to work and pretty much managing, but in the evenings and at weekends I was either being held at the police station detained on a section 136 or in A&E. No one in the police station or in casualty could understand that such a seemingly together person who had a good job could also be so destructive and wasting their resources. I was leading a completely parallel existence. Eventually,

because I was so exhausted I started to struggle at work. I took sick leave, never to return.

As soon as I gave up work, which was the only thing holding my life together, I deteriorated rapidly. My self-destruction increased to two or three times a day, I didn't sleep or eat, and my finances were in chaos. My whole life became a constant game of Russian roulette. Although I struggled with suicidal thoughts, most of the time I didn't actively want to die. I just wanted to feel safe and access help, but equally, if I died by accident as a result of what I did, I didn't care either. Let fate decide.

Eventually, this led to a point where I was admitted to hospital and was diagnosed with borderline personality disorder. I was an inpatient for 8 months. At first it was a difficult admission as my determination not to be medicated left the staff struggling to meet my needs. I did, however, manage to build up relationships with some of the more experienced staff. They helped me feel safer and they had the skills to work psychotherapeutically with me. This I found more helpful than the interventions of less inexperienced staff who tried to control me and my emotions by becoming more authoritative. This tended to escalate situations.

The team was split between those who were more open minded about working with people with borderline personality disorder and those who felt I shouldn't be treated in hospital. This was difficult for me to deal with at times as it always came across as a personal rejection. Eventually, as my ability to build relationships and to learn to trust and ask for support increased, I gained more respect from the team as a whole. This improved consistency in their approach, helped me feel that staff responses were more predictable, and this in turn helped me to feel able to trust them and ask for help, rather than self-destruct.

Throughout this time, as well as receiving support from the nursing team and psychiatrist, the work with my clinical psychologist continued. I was able to make much more progress in an environment where I felt safe. We continued to work primarily in a psychodynamic/attachment orientated way, however, some inputs from cognitive analytic approaches were very helpful in understanding the cycles and patterns of behaviour in which I would get entangled and would lead to self-destruction.

Despite the progress I made during this lengthy admission, I didn't feel that I was yet at a stage where I could survive at home again. I had a mortgage, which made options such as supported housing feel too impossible, and I still didn't trust new people enough to have care at home. A therapeutic community was therefore suggested and after some consideration and a couple of meetings with the community's outreach team, I decided that it was probably the best way forward and a step that I now felt ready to make.

This transition was probably one of the hardest I have had to make: I was leaving the safety of the hospital and was going to have to interact with peers and to survive without parents in any form. However, the therapeutic community, although difficult, proved to be the right move. Its combination of different treatment approaches, group therapy and its emphasis on residents taking responsibility for running the place and for each other, meant that I became more honest with myself and others about how I

Experience of care

was feeling, making it easier to identify my emotions and access the support I needed. It also allowed me to do what I hadn't got around to in hospital – linking my past story with my current patterns of behaviour. I saw for the first time how much my current thinking, interpretation and behaviour replicated my past survival methods in the family, and how these strategies I used as a child could no longer work as an adult. I recognised the need to learn new skills and although it sounds a simple process, the reality was that it was difficult and at times traumatic. I had to face up to the fact that, at times, I could be selfish, blame others for things that were my fault, and shut others out. I had to learn to accept all facets of myself and piece those functioning and malfunctioning parts of myself back together so that I could start to build a sense of self.

Another important thing I learned at the therapeutic community was that I needed to be my own parent and that I had the skills to do it. I had to look after myself in the way I wanted to be looked after. This would help me feel better about myself, increase my sense of self-agency, which in turn would further strengthen my sense of self. Perhaps most importantly, I learnt to interact socially. The therapeutic community gave me an environment in which I could learn what was acceptable and unacceptable in terms of dependency, and through the process of seeing my behaviour mirrored in the other residents, I realised the negative impact I could have on other people. After a year, when I came to leave I felt like I was functioning better than I had functioned in my entire life.

The difficulty for me was maintaining this once I had left the therapeutic community. Living a few hundred miles away I couldn't make use of the outreach services that easily and I was too far from the friends I made there to have regular contact with them. This meant that when I left I was socially isolated again, having not had much of a social network prior to my admission into hospital. I was also living on my own for the first time in 2 years, and dependent on the mental health services to fill the gap the therapeutic community left behind.

I continued to work with the clinical psychologist and psychiatrist I had prior to the therapeutic community, but in addition, I also had a community psychiatric nurse (CPN). I found it difficult to work individually again after group work and I also struggled with my relationship with the clinical psychologist. Having been dependent on her before, I wanted to manage the relationship in a different way using what I had learnt at the therapeutic community. However, we both found this a difficult change and consequently we struggled to find the same engagement and level of work we had achieved previously. In hindsight, this was probably one relationship I shouldn't have gone back to, but we both found it difficult to end the relationship and we got stuck in an unhelpful dynamic for a while.

The therapeutic work, at this point, came mainly from my psychiatrist, who prior to the therapeutic community was too 'advanced' for me to engage with for any more than just a general overview of my care. However, my improved ability to articulate my feelings meant that I could now engage with him therapeutically. In my community psychiatric nurse, I had a more general support that was whatever I needed it to be. This ranged from the practical and the therapeutic to the social (as much as it could be

within the boundaries of the therapist-client relationship). This flexibility was hugely helpful, especially combined with the consistency and continuity in my care I had received before and after the therapeutic community.

Unfortunately, the lack of any social network and the loss of confidence caused by my disintegration and lengthy hospital admission meant that I struggled to build on the progress of the therapeutic community. Although I was managing more than I wasn't managing, I began to self-harm again, having previously resisted this urge at the therapeutic community. My CMHT helped me to keep this to a minimum by increasing visits at times of need when I asked for help and through short hospital admissions (2 to 3 days) where I could have some respite and feel safe. Also crucial in helping to keep self-harm to a minimum was the social services out-of-hours team. Although I had used this service before the therapeutic community, the calls would often escalate crises as I struggled to accept that at that time they couldn't meet my needs. However, now that I could articulate myself better and wished to use alternative methods to cope, I established good relationships with most of the team. The out-of-hours team were happy to engage in supportive conversations as long as they had time, and if they didn't they would explain that to me so that I wouldn't feel personally rejected and agree to ring me back when they had more time. This worked really well for me, as my most difficult times were at night and their consistent and predictable responses were helpful in settling me ready to sleep (with the aid of promethazine at times). This non-judgemental response allowed me to engage enough to articulate what I was feeling and to move away from the feelings (often onto mundane topics for a short while) until I felt calm enough to manage the rest of the night. Knowing that this service was there and that there was always an option to ring back made it such a huge part of my progress after leaving the therapeutic community.

All of this helped me to maintain a much higher level of functioning. However, my lack of confidence prevented me from making much progress in the other areas of my life. I was still a full-time patient and I struggled to believe that this would ever be different. Since the therapeutic community I had found the label of borderline personality disorder a hindrance. It made me feel like a second class citizen, like I could never be normal. I struggled not to believe the myth that it was untreatable and felt that no one would want to employ me.

Despite the progress I had made, I couldn't live with the thought that my life would always be limited. I sank into a depression, and this combined with the unfortunate timing of another rape, a destructive relationship as a way of coping with the rape, a pregnancy as a result of the destructive relationship and subsequent termination, and the retirement of my psychiatrist – all in the space of about 8 months – destabilised me so much that I ended up being hospitalised involuntarily under Section 2 of the Mental Health Act.

Although, at the time, this appeared to be a huge setback, this admission changed a lot for me. I was prescribed an antidepressant (mirtazapine) for a few months which I found really helped to lift my mood. However, towards the end of the admission when my mood had improved, I also realised that I had to make a choice – to live my life, reject the label's myths and decide for myself my limitations, or to believe the

Experience of care

myths and accept that I would be a patient for the rest of my life. The latter was not an option to me, so after I came off the section I decided that I needed to face my fears and start to rebuild my life. I decided to enrol at university to undertake a degree in psychology. This proved to be a successful move, and one that gave me a good balance between commitment and space for me to manage myself and the transition I needed to take me from being a patient back to being a functioning member of society. It also allowed me to gain confidence in an environment that didn't ask too much of me most of the time. It enabled me to get to a point where I had a social network, an identity other than 'patient' and feel able to leave behind my last connections with the service, my community psychiatric nurse, and the social services out-of-hours team.

I did it – I am no longer a patient. I completed my degree, and am managing to work full-time. I no longer consider myself to have a diagnosis of borderline personality disorder. I have none of the symptoms and when I look around at other people I don't seem to be any different from anyone else. The only time I feel different is when I recognise that my journey to this point in my life has been a lot more complicated than many people I come into contact with. However, when I look around I also see myself handling situations more competently than many other people. I have gained in strength and resilience as a result of my experience of handling such intense emotions, which means that I am not easily overwhelmed by life's challenges. I'm not perfect though. I still have bad days, but talking to friends, so do most people. I really am no different. I no longer have thoughts of self-harm. My moods are more recognisable as normal, and my sense of self is much stronger and doesn't fragment anymore. In addition, I am more open, and able to recognise, contain and talk about my emotions. I can also manage friendships and intimate relationships. The only thing that is remotely borderline personality disordered about me now is that I can still remember how it felt to be that way – but it is just a memory.

4.2.4 Personal account C

For me having borderline personality disorder is having constant and unremitting feelings of unbearable and overwhelming sadness, anger, depression, negativity, hatred, emptiness, frustration, helplessness, passivity, procrastination, loneliness and boredom. Feelings of anxiety are like silent screams in my head and it is as if masses of electricity are channelling through my body.

I feel unloved and unlovable and constantly doubt that anyone likes me or even knows I exist. Both my body and mind feel like they are toxic and polluted. I always felt dirty and scruffy no matter how many baths I take. My sense of physical self is constantly changing – I am not sure what I look like and my facial features keep changing shape and getting uglier and uglier. Mirrors are terrifying – I always think I'm fatter or skinnier than I am.

Sometimes it seems like people are sneering and laughing at me all the time and attractive women look at me like they are murdering me with their eyes. Other times it is as if I am invisible. At times I hate everyone and everything. Ideas about who I

am and what I want to do fluctuate from week to week. My perspectives, thoughts and decisions are easily undermined by what other people think or say and I often put on different voices to fit in. I am never satisfied with my appearance, but then I am never satisfied *per se* – perfect is not even perfect enough.

My feelings lead me to self-medicate with alcohol and food and to overdose. I slash my arms, chest stomach and thighs with a razor blade and constantly think about killing myself or visualise my own death.

I have also had some hallucinations, such as the devil's face appearing on the wall and talking to me in Latin and the devil coming into my flat in the guise of black poodle and me putting my hand inside its body. I also have headaches, panic in my stomach, and feel sick and tired all the time. It is often hard to get to sleep and I have horrific nightmares.

Signs of my emerging personality disorder started in early childhood in the late 1960s/early 1970s. I was so disturbed that my adoptive parents had to put bars over my windows because I used to throw all my toys and bed linen out of the window every night and they were worried I would fall out.

I was told that I was adopted very early on and have no memory of ever thinking that my adoptive parents were my real parents. As far back as I can remember I used to pray that my real Mum would rescue me. When someone came to the front door I used to rush towards it shouting 'Is that my Mum? Has she come to get me?' My Nan remembers me asking women in the supermarket the same questions. I also pleaded with any women teachers from infant school upwards if they would adopt me.

I was a very disruptive, naughty child who wanted so desperately to be loved and accepted by my adoptive family. I had behavioural problems and used to rock backwards and forwards going into a trance-like state for hours everyday. I had terrible insomnia from early on and would repeatedly bang my head on the pillow and make a droning noise to distract myself from the unbearable agitation that I felt. This behaviour ignited a cycle of physical and emotional abuse at the hands of my adoptive parents who did not understand the mental distress I had to endure on a daily basis. My father, exasperated that he couldn't sleep because of my head banging, used to come into my bedroom and punch me until I stopped. I used to have dreams where the devil would tell me to go into my parents' bedroom and smash my Mum's and Dad's heads in with a hammer.

My father was a rigid disciplinarian and I quickly became the black sheep of the family – the source of all the family's woes and misery was my fault. I spoilt everything. I was to blame for everything. They went on family days out and I was excluded for being naughty, locked out of the family home, and left sitting in the back garden on my own for hours on end until they returned, happy that they had had a fun day out without me around to spoil it for them. I used to deliberately say all my Christmas presents were a load of shit to annoy them and ceremoniously smash them all up on Christmas day in utter defiance then eat as much chocolate I could until I threw up. I often spent Christmas day banished to my room.

I was a habitual liar at school telling my friends that I went on amazing holidays and had all these amazing toys (when the exact opposite was true). My father often

Experience of care

withheld presents and instead gave them to my brother and sister to punish me. To punish me further he refused to fund school trips and would ration the sweets my Mum bought me in an attempt to control my behaviour.

I started to dress in increasingly attention-seeking clothes. I used to bite my nails down so far they would bleed and were very painful and as a punishment I was told my pocket money had been stopped for 5 years.

One time after I refused to rake the back garden my father beat me with the rake. I ran into the kitchen hoping my Mum would protect me but she grabbed me so that my Dad could beat me some more. I grabbed a carving knife and tried to stab her so she'd let me go. I was beaten severely for this and after that they contacted social services requesting I be put in a home for maladjusted children. I was 12 years old. Social services tried to work with the family to overcome our problems but my parents refused to attend the therapy sessions and I had to go on my own. When the decision was made not to send me away my father was so angry he just used to act as if I didn't exist. The rest of the family tried their hardest to get on with their lives but the silent aggression from both sides made me run away and spend hours on my own in the woods reading my comics. It was during this time that I started to feel suicidal and constantly tell my Mum and Dad that I wanted to die to which I was told that I had growing pains.

I hated my Mum and Dad and wished they were both dead and constantly spat on their food and urinated in their drinks if I could get away with it. I used to bully my younger brother because he was their flesh and blood and mercilessly beat and threatened both my brother and sister until they cried and begged me to stop. I started to set fire to things and torture insects. I prayed to the devil that people I hated at school would be killed in horrific accidents and I used to steal from my parents and smash my brother's and sister's toys to punish them. I remember watching the film 'The Omen' and thinking that I was the Antichrist.

I was very disruptive at school and repeatedly got the cane for verbal attacks, such as calling the headmaster a 'cunt' to his face in assembly. Even at junior school when I was 10, my father told me to tell my teacher she was a 'stupid bitch', which I did and got into a lot of trouble.

By 14 I had started sniffing glue to escape the misery I felt and also experimenting with cross-dressing. I was often sent home from school for wearing women's clothing. I started to alienate the few friends I had by doing this but I thought I was the messiah and they would all worship me one day.

My father hated my emerging transvestism and smashed my make-up box to pieces and forbade me from wearing any women's clothing around the house. The threat of being thrown out onto the street was made constantly. I went on hunger strike and stopped swallowing my food. I used to store all the rotting mouthfuls of half-digested food in shopping bags in my wardrobe.

I left school in 1983, failed to get into college and was on the dole for 3 years. During this time my eating disorder worsened and I developed severe acne. I drifted through the 1980s in a haze of solvent abuse and, due to my terror of women, found some relief in pretending to be homosexual.

The slow decline into hell that started in my childhood gathered pace during my twenties. I had one serious relationship with a girl but it was stormy and complex. I used to feel nausea after sex and constantly behaved like a homosexual and lied about my sexuality to her. In relationships I have an intense need for constant reassurance; and when I try to hold back I get unbearable feelings of panic and fear of imminent abandonment. I also find it very difficult to trust people.

I went through a particularly intense stage of religiosity in 1988 when I became a Jehovah's Witness but I very quickly started to feel disconnected from everyone in the congregation and habitually fantasised about murdering and torturing them.

When my relationship ended I stated to drink heavily and self-mutilate, which led to my first contact with mental health services in 1990 at the age of 23. I saw my GP first, who referred me to a consultant psychiatrist. After three lengthy assessments I was told I had symptoms of a classic disorder, but that it could not be treated with medication. I was formally discharged from services never knowing what the disorder was. Because of this I believed that the psychiatrist thought I was making it up.

My parents, who were divorced by this time, had the sense that because I had been discharged there mustn't be anything that wrong with me. My parents and friends also thought I was making it up. I was left thinking that my problems were not real even though I constantly felt suicidal and my behaviour by that time was very extreme. People thought I had mild depression or was just an attention seeker. I was put on an antidepressant by my GP. But my depression, drinking and self-harm worsened and I constantly spoke of suicide. After I did try to commit suicide in 1991, I was given ten or 12 1-hour sessions of CAT. This made no difference whatsoever and I continued to deteriorate. In 1993 after another suicide attempt I had 20 sessions of CBT but this also did nothing to help me.

In the early 1990s I was reunited with my real parents. This was not without problems. After I was told that my mother attempted to have me aborted I started to despise her and fantasise about murdering and torturing my real parents as well. I particularly hated my real sister.

I endeavoured to try and reconnect with them in 1997 after I was made homeless and had been living in a drug psychosis unit for 13 months because there was nowhere else to put me. I didn't have psychosis and often wondered why I was allowed to stay there. It was during that time that a junior staff member broke her professional boundaries and told me I had borderline personality disorder. I misunderstood what she had said and thought it meant I was on the borderline of having a personality disorder, and therefore was not that serious (even though I felt suicidal all the time).

After this I was housed in an old people's block on my own and rapidly spiralled out of control. I used to over-medicate with all the drugs I was taking: I would take a cocktail of SSRIs, sleeping tablets and alcohol that would make me go into a trance. I used to do this on a daily basis and just lie in bed all day in a haze rarely getting dressed or leaving the flat. I couldn't look after myself and lived off the same meal everyday: cornflakes, saveloy and chips.

My flat was undecorated and I slept on a mattress on the floor. I was obsessed with perfection and spent hours redoing the same small DIY jobs over and over again

Experience of care

compelled by a vision of my dream home. In reality I was living in an uncarpeted, unfurnished flat with no furniture and which was covered in plaster dust from my endless attempts to make all the walls perfectly flat and smooth.

In the late 1990s I had a few therapy sessions for body dysmorphic disorder, but the therapist seemed very under-trained. She was a nice person and seemed to care, but she said that everyone has a personality disorder. She used to give me photocopies from books to read which were of no benefit whatsoever.

The thing that finally had an impact on my symptoms was attending a therapeutic community from 2005–2006. It enabled me to make some progress, to understand myself, understand boundaries, and to see the effect my behaviour had on others (a massive deterrent). I was able to start loving myself and to have respect for myself and others. It also allowed me to break my dependency on my real mother, to gain insight into my cognitive distortions, to learn how to make and keep friends, how to manage destructive impulses and to ask for help. I had not realised that the label ‘personality disorder’ was so stigmatising until I went to the therapeutic community and met other sufferers. However, even with a year in intensive therapy at the therapeutic community I still have only improved in some areas and will need ongoing support and help and further treatments.

After I left the therapeutic community, my consultant psychiatrist was advised that I should remain on an enhanced care programme approach, but he ignored this advice and withdrew my access to a CPN and the self-harm team. In my first outpatients appointment after leaving the therapeutic community he angrily raised his voice and told me ‘there was no scientific evidence to show that you will ever improve’.

Borderline personality disorder has had a serious impact on my life. I can’t concentrate for very long and I get confused by what people mean. My obsessions about perfection get in the way of doing almost anything practical and I can’t complete tasks. Although I crave perfect order I live in total chaos, with rubbish, clothes, crockery and magazines strewn all over the place. I live in absolute squalor and never have any motivation to tidy up because attaining perfection is so stressful I don’t even want to try. I am unable to make plans and keep to them and I find it almost impossible to make decisions. I get bored and agitated very easily and thoughts go round and round in circles in my head. To most people boredom is endurable, but when you’ve got borderline personality disorder boredom is a killer. You’re too unmotivated and hate yourself so much that you don’t want to do anything, go anywhere or see anyone. Boredom will make you self-harm and start that fever pitch agony of wanting to commit suicide. I have hair trigger explosions of intense feelings. Sometimes I feel so excited about doing something it’s as if I could conquer the world then a couple of hours later it just seems like a load of bollocks. I can’t decide what I want to do with my life. I find it difficult to work unsupervised and I have started college courses but then I get angry with the other students and end up hating everyone, giving up and lying in bed all day for weeks on end.

It has also seriously impacted on my relationships and I find it very difficult to make friends. I feel angry that people don’t understand me and in turn people are frightened by my rages. I am terrified of engaging in conversation people I’ve not met

before because I am worried they will think I'm boring. I love people one minute and then hate them and want to hurt them the next. Likewise, I can fall in love with someone almost instantaneously then be repulsed by them in a matter of hours. I can be abusive then feel terrible remorse and fear being abandoned. I have sexual feelings but can't have sex; this drives me insane as the hunger never goes.

My condition has changed since leaving the therapeutic community but not as much as I'd hoped. Some of the feelings are not as extreme as they were before I went there but because I refuse medication some are even worse. I'm learning how to deal with them better but I still relapse and battle with suicidal and violent feelings, and my obsessions around perfectionism are still really bad. I still self-harm but realise it is futile and I am alcohol dependent; if I'd got some help when I left the therapeutic community I would not have started self-harming or drinking again. I also have problems cooking and looking after myself. However there are some days when I like who I am more than ever and I feel happier than I ever have done in my entire life. I am also in a relationship, which although is a bit unhealthy at times, is not as co-dependent as in the past, and I have made improvements to make it work.

In order to try and stay well I reassure myself and assume that things will be positive. I relax more and meditate on what I want and not on what I don't want. I have a gratitude list of all the good things in my life that I read when I feel bad. To help with my self-esteem I try to take a pride in my appearance. I attend Alcoholics Anonymous, which is helpful although I find the interactions with other alcoholics can be problematic at times. I try to be more 'boundaried' with my emotions and read as much as I can about personal growth and recovery to give me hope. I keep myself busy and avoid people and situations that wind me up. I also try to have contact with other people recovering from borderline personality disorder at least once a month.

4.2.5 Personal account D

I don't know when I was first diagnosed with borderline personality disorder, but the first time I knew about it was when I read it on a report, about 5 years after I had been initially referred to psychiatric services. I was totally horrified and ashamed. I thought I was one of the 'untouchables', one of those patients I had heard described as untreatable and extremely manipulative by health professionals whom I regarded as highly competent. I fell into deep shock and crisis for some time after.

When I was a young child I was over-sensitive and needy, constantly acting out for attention. Unfortunately both my parents were ill-equipped for parenthood: my father was an alcoholic and my mother had her own mental health problems and never even wanted children. Early on I became the runt of the litter, constantly bullied and shamed, so I learnt to trust no one and keep to myself. This was an impossible task for someone with my personality.

At age 32 after having been severely bulimic for many years, and still not having managed to kill myself, I sought psychiatric help. This was initially an eating disorders

Experience of care

unit. The staff there were very kind, but I always felt that they didn't know what to do with me. I felt like I was disintegrating.

I had two stays in the eating disorders unit with the second being followed by 5 months in a drug and alcohol rehabilitation unit, all of which helped regulate my behaviours. But without my usual coping strategies (alcohol, drugs, food and cutting) I had no way of surviving what felt like such a cruel and dangerous world. So despite doing everything I had been taught for a while, and despite all my determination to be well, I eventually succumbed to my old ways of coping. As all my treatment had been aimed at stopping them, I fell back into the bottomless pit of shame and disgust, only to then be forced back into hospital or a crisis unit for a short respite. My stays in both the hospital and the crisis unit were invaluable at those times of crisis, because they were time-limited and managed appropriately. As I had a strong need to be looked after and be rescued from myself, it was essential for me that it was like this.

But despite weekly psychotherapy, and regular appointments with several different health professionals, none of it was getting to the root of the problem and my admissions were becoming more frequent. So eventually I was admitted to a specialist day unit for borderline personality disorder. Here for the first time I was not looked at as a set of behaviours and stuck in an appropriate box. Instead I was seen as an individual with my own problems that staff wanted to learn about and help me with. Finally I felt listened to and understood as people could see me as a whole set of problems rather than looking at the individual bits of me. During my time at the unit I learnt that I use what others see as unhealthy coping strategies; to some extent they work for me and they are what I have known for almost 30 years. There are times where I do fall back on them because life can feel just too painful and frightening without them. I use them as my armour to protect me from the outside world. So my goal changed from giving up all these behaviours to minimising them instead and not to shame and humiliate myself when I once again fell back on them.

My relationship with my psychiatrist is very good and I trust him implicitly as he has always tried to understand, and has always been totally reliable and consistent. I also know I can contact him between appointments if I am not able to cope and he will try to see me. This gives me a lot of strength and so reduces the need to contact him as a result.

I have also been one of the lucky few who was in the first instant referred to my local hospital, which has very good specialist services such as dual diagnosis, an eating disorders unit, a crisis unit and specialist psychotherapy services for borderline personality disorder. But I was plagued by long waiting lists and being passed from one health professional to another until I was given the right treatment.

I have always tried to find support groups to help myself as much as possible and help me through the gaps in between appointments. I have found these invaluable and very supportive, even though I felt there was a big gap between other people's problems and my own.

Borderline personality disorder affects my entire life, from the minute I get up to the minute I go to bed, although to a much lesser degree than it used to. But all day I have the misery of sitting in my flat by myself everyday because the fear of being with

people is still greater than the fear of being alone; the sleepless nights and tired days, so that I can only work a few hours before feeling exhausted; the continual racing mind and appalling concentration, which makes conversations hard to follow; and feeling battered and hurt constantly by people due to my over-sensitivity. But on the worst days I'm learning that the safest and kindest thing I can do for myself is to climb back into bed for the day until the suicidal thoughts abate.

I'm learning to live life, which is often filled with pain, fear and mental torture, but I'm also learning that some days are better than others. I'm learning to accept my fragilities: that there are many everyday things that feel impossible to me, as well as many things that I do to myself in the secrecy of my flat that others would be totally appalled by. It all seems manageable so long as I don't compare myself and my mess of a life with others.

With no close friends or family and only razor blades, food and alcohol as my allies, I guess borderline personality disorder continues to be my only close friend.

4.2.6 Personal account E

I am the biological mother and carer of my son, who has borderline personality disorder. He was adopted and when we met in 1991, when he was 24, it was obvious he had some kind of mental health problem. In 1990 he was referred by his GP to a consultant psychiatrist at his local community mental health service. Suffering with obsessive behaviours, social phobias and eating problems, the final straw came when making an item of clothing and he had totally lost control. After several weeks of assessment he was told he had symptoms of an unnamed classic disorder that could not be treated with medication; the consultant told him there was nothing more he could do for him and discharged him from his care.

Once I got to know my son he eventually told me about his obsessions concerning his body and clothes, his aggressive thoughts, and his drinking and self-harming. He told me that when a relationship with a girlfriend had ended he made massive cuts with a razor on his chest and arms and put bleach in them. He covered up his initial self-harming episodes and he was left with hideous scars. He also told me about his physically abusive childhood and lack of emotional bonding with his adoptive parents.

I was beside myself with grief, appalled that nobody seemed to care enough to help or listen to my son's very distressing story. He went back to his GP who gave him antidepressants and arranged a course with a local counsellor. Looking back now this seems to me to be wilful neglect as he fell deeper into an abyss of misery.

This was all new to me but at that time I felt sure that with my support and further help there would be a light at the end of the tunnel. However, I watched him deteriorate even further over the next 5 years with no real support or constructive treatment from his CMHT. His adoptive mother couldn't cope with him and in 1994 when she decided to sell the family home she told him to leave. This threw him into total chaos and bouts of extreme anxiety and excessive anger, which he turned in on himself. At this time he seemed to draw away from me and for about a year had only spasmodic contact.

Experience of care

He seemed to find some solace in the fact he was given a social worker who seemed to be trying to sort out his life for him while finding a place for him in a hostel.

The hostel was for people with schizophrenia and those with drug psychosis. He received no treatment and had only spasmodic visits with a consultant psychiatrist when in crisis. I felt totally helpless for the next 2 years as I watched an extremely intelligent and articulate young man with real creative talent living a distressing life, cleaning toilets to earn money, having no social life, taking antidepressants, drinking to excess, self-harming and attempting suicide by taking an overdose and slashing himself severely.

During this time my son learnt from a female member of staff at the hostel that he had borderline personality disorder. This was a lapse on her part and totally unprofessional, but at least we now knew. We mistakenly assumed it meant that he was only on the borderline of something, not having a full disorder, so we didn't really see it as that serious. Nobody told us any different and we were left floundering in the dark.

I could not bear to see my son suffering at the hands of his local CMHT any longer so in 1996 I asked him to stay with me temporarily and offered him some work in my office, which was a creative environment, just doing simple tasks that would keep him occupied. His care was transferred to our local CMHT under the care of a consultant psychiatrist. I remember thinking that at last, with a new mental health team, we had hope, we would be able to access better treatment and perhaps begin to understand what was really the problem. With my love, care and support and real treatment I thought I would see my son at last living a life he really deserved.

What followed then was the most traumatic 10 years of my life. The glimmer of hope we had at the outset was soon to be extinguished. The local trust was worse than my son's previous area. The people who had been entrusted with his care treated him with neglect and total disregard for his feelings yet again.

My son has been given so many diagnoses: in addition to borderline personality disorder he has been told he is body dysmorphic, schizotypal, schizoaffective and obsessive-compulsive. Sometimes when I asked the consultant for more information he denied he had even given that diagnosis – he changed it so many times he couldn't remember what he had said. The consultant never explained anything in great detail, all he seemed to do was prescribe medication and tell us both to be patient. He told us that the local trust was running with restricted budgets and staff and that there were no trained therapists because of maternity leave. He took months to follow things up and lost important letters. I complained there was no CPN but the consultant said there was no need for one. My son was never taken seriously and was told on numerous occasions when expressing his feelings of suicide that he didn't feel suicidal and should stop saying it. His anger grew and grew.

My son also met with no understanding from others, such as nurses who attempted to stitch his cuts with no anaesthetic. He was handled roughly, without any sympathy or care, and with an attitude of 'Oh well, you did this to yourself'. Usually when he was discharged we would go home with him caked in dried blood because nobody had

bothered to clean him up. On more than one occasion I came home to see a noose hanging from the banisters and blood everywhere.

I also complained that my son's social worker was hardly ever available, especially in a crisis. She curtailed and cancelled appointments and gave him misinformation about housing. On one occasion when I was stressed and just couldn't take anymore I took my son to the CMHT and wanted to leave him there. All the staff did was leave us both in a room and kept telling me there was nothing they could do, our consultant wasn't available and to go home. In the end when I had calmed down I did go home, feeling totally defeated and completely alone.

Around 1998 it seemed that body dysmorphic disorder was the main diagnosis. A friend of my son heard of a specialist in her area and found out we could see him privately. After seeing my son the specialist agreed that his condition was extremely severe and needed lengthy inpatient treatment. He did not agree with the drugs regime he had been given – a cocktail of antipsychotics, mood stabilisers and antidepressants. However, it was a private clinic, and while they had some funding arrangements for some NHS trusts, this did not include ours. We would have to fight for a place – and fight I did. We were told by our trust that it was procedure to apply to a hospital that the trust had connections with; if they denied him access to their programme then he would automatically get funding for the private clinic. This process took over 2 years, with much prompting and demands from me. After waiting a very long time for an appointment at the hospital and being told that they could not give him the 24-hour support he would need, they said that the private clinic would be the best place for him. I felt so relieved that at last he would get treatment from somebody who really understood him. But soon our hopes were dashed again. The hospital changed their criteria – there was inpatient treatment available after all.

My son was in total despair about this, which led to more self-harming and further suicide attempts.

During this period my son lived in total chaos even though I tried on a daily basis to help him cook and tidy his room and to learn coping strategies. One time I came home to find the house had been totally trashed, windows broken, furniture thrown outside, and armed police at the property asking if I wanted to press charges. The house was full of blood. He was sent home the following morning and there was no visit to assess if he was a danger to himself or me. With all this aggression it was obvious that the inability to be heard was growing and growing, but nobody was listening.

Because of the above episode my son was sent to see a forensic psychiatrist. She assessed him and wrote a report. We were not allowed to read this at the time, although when we subsequently made an official complaint we did see the notes. In this report the consultant said that my son was a danger to me and that it was in his best interest not to live with me. And yet they allowed us to live together for a good many years after this episode. He was becoming more and more dependant on me and would have anxiety attacks if I were ill or had to travel any distance in my car. He was afraid that I would not return or die.

Experience of care

On another occasion when I had gone to bed, he tried to kill himself with exhaust fumes from my car. Luckily the car was parked on a public road and someone banged on the window. He came staggering into my bedroom and dropped unconscious to the floor. As I waited for the ambulance, I held him in my arms and remember thinking that he was going to die. The ambulance staff were very supportive and caring but at the hospital it was seen as just another suicide attempt, and he received no sympathy.

I just had to keep going, keep working, and keep looking after my son. I was the only one who seemed to care. I wanted to scream from the rooftops, 'SOMEBODY HELP US PLEASE'. But I was also beginning to resent having my son living with me. I began to see my son as the disorder and forgot that it was an illness, but his behaviour around the house and in my office was becoming intolerable. I was totally overwhelmed by the enormity of it all – I was trying to run my own business, pay all the bills and single-handedly (I had separated from my husband) cope with my son's mood swings, self-harm and aggression. I begged his social worker to find somewhere for him to live apart from me and she told us she had found him a place at a shared housing scheme. He was shown a room and felt quite happy about it, but then we were told they could not accept him because he had borderline personality disorder. One would have thought that a social worker, working in this area with vulnerable people, would know this. So yet again his hopes were raised and then dashed.

By this point, as the social worker knew, my relationship with my son was very strained. We began to argue all the time, and I went from being an outgoing and fun person to someone who didn't sleep, was very tearful and extremely stressed. Like my son, I felt I was going down the same path of wanting to give up – I wanted to climb into bed and never wake up. I was assessed for carer support, but I didn't need money – I NEEDED TREATMENT FOR MY SON. If someone had taken us seriously I feel we would never have been allowed to get into this awful situation. In the end I saw a counsellor whom I found and paid out of my own money. In all these years I have had no support whatsoever. I was not told how to deal with personality disorder; all I have gleaned is through books that I have found by searching on the web and purchasing myself.

Finally in 2004 after several failed attempts of gaining appropriate treatment – which included brief and ineffective sessions of CBT with poorly trained therapists whose expertise extended no further than a cup of tea and a chat and giving him photocopies from books to read – and continued episodes of self-harm and overdose, his consultant psychiatrist, who had expressed his own frustration that my son wasn't making progress despite the fact he had never been offered any significant inpatient treatment, informed us in a very offhand way that 'there may be somewhere that can help you, we have just sent someone here, just don't know what else to try, this is the last thing'.

This 'last thing' turned out to be a therapeutic community run on democratic lines for people with severe personality disorder. After several agonising months of waiting my son was accepted in the summer of 2005 for the year-long programme.

We have found out since that the CMHT had in fact been sending patients there for a number of years and that it did not cost them a penny. This infuriated us because my

son was told he could not access treatment due to local PCT funding issues. I feel that the consultant wasted a good 10 years of my son's life through ineptitude and prejudice.

The therapeutic community helped my son to gain a sense of who he is and work through the pain of the abuse he suffered as a child. This was something he was never allowed to express in all the previous years because his consultant psychiatrist said it wasn't good to go over the past. It was a very challenging regime but it is a testimony of his will to succeed that he got through the year at the therapeutic community. I am very proud of him. My son's stay there changed his life for the better and immediately after his release he was extremely hopeful. For the first time since I had known him, I could hear his enthusiasm and optimism for life loud and clear. He was confident, had self-esteem and made plans for the future, registering at our local college for a course, working towards some qualifications in art therapy. I was so delighted and relieved that at last, at the age of 40, he could begin to lead a better life.

In that year I also went into therapy, which I continue to this day and have funded by myself, to try and unravel what had gone on in those past years, to come to terms with my son's adoption, his abuse by his adoptive parents and our relationship. I slowly began to get my life back, and to understand what my son's diagnosis actually means. I have read and researched so much and I have made new friends and been happier than I have been in years. Above all I have learnt to make boundaries, which I have tried hard to stick to since my son's discharge. This has led to my son having a lot of ill feeling towards me, which I find very distressing. However in therapy I am learning to deal with this. I can only hope in time he will come to see that the decisions I made about him living and working independently from me will serve him better in the long run.

So finally there seemed to be a light at the end of the tunnel, but we were proved wrong.

The therapeutic community offered outreach support, a weekly meeting held in London for 6 months, and they also put together a care package of support to help my son through the initial release period and help him sustain the massive gains he had made. They liaised with our local CMHT and consultant psychiatrist, and his CPN (whom my son had not met before) attended two CPA [care programme approach] meetings to make sure everything was in place prior to his discharge and ready for his aftercare. They advised his consultant that he should remain on an enhanced CPA to help him through the initial period post-discharge. But in their ignorance they denied him this, withdrew the CPN in the first week after he left the therapeutic community, and said that my son had made improvements and lowered his CPA level. He was not given a key worker or social worker. He was denied access to an emergency phone support network and told to make an appointment to see his GP if he felt suicidal. We tried to complain and saw our local MP in the hope that his intervention would effect a turn around. The staff at the therapeutic community requested a meeting with the CMHT to try to persuade them to reconsider their disastrous decision to ignore their recommendations. This was immediately refused and the week that my son was discharged the CMHT told him they no longer wanted him on their books. They said

Experience of care

that because of his improvements they had nothing more to offer. The consultant even challenged the legitimacy of personality disorder as a real diagnosis telling my son he had to look after people with real mental illnesses and that there was no clinical evidence that he would ever fully recover. My son requested another consultant, but this person said the same kinds of things.

Since then my son has floundered. He has started to drink and self-harm again and last summer took a very serious overdose. He gave up college because his diagnosis leaked out and certain members of staff started to treat him differently. He fought extremely hard against all the odds to keep going without medication and with the support of the friends he made at the hospital.

Then we found out that at the time he was discharged from the therapeutic community the PCT had set up a personality disorder community support project about 10 minutes' walk from my son's flat. The CMHT had failed to mention this even though in a meeting with the therapeutic community they were asked if any such services were available in the area, as the therapeutic community was aware that at that time PCTs were being given funding to set them up in most areas. To date my son has not been offered a place there. At one time he paid to see a therapist for weekly sessions at a local counselling centre; when he told them of his diagnosis the therapist terminated the therapy.

I was trying to keep to my boundaries of supporting him to live independently but the fact that he was receiving no support from the local CMHT only made me feel compelled to help. This was driving him back to me, something he didn't want, but there was nobody else. All the professionals have advised us about us keeping healthy boundaries, which we have tried to do, but it's extremely difficult for my son who has no network of support. He has the friends he made in the therapeutic community, but sometimes this only adds to his anger and feelings of neglect because they live in areas that offer far more support. If he had received help and support from the appropriate channels I feel our relationship would now be stronger. However, it's falling apart because he feels I neglected him when he needed me.

Recently he has been offered 12 weeks of therapy by the head of the psychology department of our local trust. We believe this is a result of our official complaint that is still ongoing. He also applied for an art foundation course at the same college but was rejected. He was told that with his diagnosis he would not cope. He ended up doing a pre-foundation course, which is so elementary that he is unstimulated by it. His tutors could see he wasn't being stretched and his talents far exceeded the basic lessons.

It seems that whichever way he turns he is blocked by prejudice and outmoded beliefs. At this present time feelings of hopelessness permeate his waking hours and his extreme anger has returned. With two recent suicide attempts I have to face the fact that one day he may take his life. This would be such a tragedy for such a loving, caring man who is torn apart and struggling without help and understanding. He wants to stand on his own two feet and is not allowed to. He was so close to having a real life and through wilful neglect he is sliding back to how he was before.

Only through public awareness and the education of professionals in all areas will people suffering from this disorder get the real help and support they need. The biggest issues for both my son and me is being heard, understood, and having one's feelings validated. I also believe that it is valuable for professionals to hear the carer's views on the disorder. With help, education and support, carers could be an even greater asset than they already are and be properly recognised for the support that they give.

My son has a long way to go and sadly has slipped back for now, but he has made big strides forward since his stay in the therapeutic community and he has the confidence to fight for his right to appropriate care and support.

4.3 REVIEW OF THE QUALITATIVE LITERATURE

4.3.1 Introduction

A review of the qualitative literature was conducted to illuminate the experience of people with borderline personality disorder in terms of the broad themes of receiving the diagnosis, accessing services and having treatment. It was recognised by the GDG that the search of the qualitative literature would probably not capture the breadth of service user experience, which may include considerable periods when people with borderline personality disorder are not in treatment. It should be noted that the qualitative evidence was limited with regards to the treatments reviewed, with an emphasis on DBT, and very little on therapeutic communities to support the positive statements made in the personal accounts above. The literature on selfharm was not reviewed for this guideline (see the NICE guideline on self-harm [NCCMH, 2004]).

4.3.2 Evidence search

In order to draw on as wide an evidence base as possible the GDG asked the clinical question: what is the experience of people with borderline personality disorder of care in different settings?

The most appropriate research design to answer this is descriptive material collected from the first-hand experiences of service users, either from one-to-one or group interviews or focus groups, or from surveys. This kind of material can either be presented in a fairly 'raw' state or it can be subjected to analysis using a theoretically driven qualitative methodology, such as grounded theory or discourse analysis.

In order to source such material, a search for published studies was undertaken which was supplemented by a search of the grey literature. The electronic databases searched are given in Table 4. Details of the search strings used are in appendix 7.

Ten studies were found that contained material relevant to the clinical question (see Table 5).

4.3.3 Diagnosis and stigma

The experience of receiving the diagnosis of borderline personality disorder and issues surrounding the 'label' and the stigma associated with it were reported by six of the included studies.

Horn and colleagues (2007) summarised the results of semi-structured interviews conducted with five service users with a diagnosis of borderline personality disorder, focusing on their understanding of the diagnosis, how they thought it had affected them, their view of themselves and others' views of them. The following themes were identified.

Knowledge as power: For service users this was both positive and negative. Knowledge of the diagnosis and professional opinions was experienced as power, both for the service user and for others. For some the diagnosis provided a focus and sense of control, for example the 'label' could provide some clarity and organisation of the 'chaos' experienced by the service user. However, for others, who had been given little information or explanation about the diagnosis (and what information they

Table 4: Databases searched and inclusion/exclusion criteria for studies of inpatient care

Electronic databases	HMIC, MEDLINE, EMBASE, PsycINFO, CINAHL
Date searched	HMIC: database inception to January 2007; others to August 2007
Update searches	March 2008; May 2008
Study design	Qualitative studies, surveys, observational studies
Patient population	People with a diagnosis of a personality disorder
Additional search terms	Health services; patient attitude, participation, experience or views
Outcomes	None specified

Table 5: Studies of service user views of services

Study	N	Diagnosis	Research design
Crawford <i>et al.</i> , 2007	Approx. 190*	Cluster B and C personality disorder	Individual interviews and focus group disorder
Cunningham <i>et al.</i> , 2004	14	Borderline personality disorder	Semi-structured interviews

Haigh, 2002	14	Personality disorder	Summary of views
Hodgetts <i>et al.</i> , 2007	5	Borderline personality disorder	Semi-structured interviews
Horn <i>et al.</i> , 2007	5	Borderline personality disorder	Summary of views gathered during semi-structured interviews
Hummelen <i>et al.</i> , 2007	8	Borderline personality disorder	Semi-structured interviews
Morant & King, 2003	15	Borderline personality disorder	Semi-structured interviews questionnaires routine clinical data
Nehls, 1999	30	Borderline personality disorder	Interviews
Ramon <i>et al.</i> , 2001	50	Personality disorder	Semi-structured interviews questionnaires
Stalker <i>et al.</i> , 2005	10	Personality disorder	Interviews with analysis based on grounded theory

*Up to ten service users and three carers at each of 11 sites, plus six service users for a focus group; final numbers not given.

were given tended to be negative), the diagnosis represented knowledge withheld and the viewing of others as experts.

Uncertainty about what the diagnosis meant. While for some service users the diagnosis led to a sense of knowledge and control, for others it was not useful and too simplistic. It did not appear to match their understanding of their difficulties, and service users were left feeling unsure whether they were ill or just troublemakers.

Diagnosis as rejection. Some service users described diagnosis as a way for services to reject them and withdraw from them. This judgement was accepted and internalised by some service users, which led to service users in turn rejecting services if they were offered at a later stage.

Diagnosis is about not fitting. Some service users felt that that diagnosis was being used because they did not fit into any clear categories. They spoke of the diagnosis as a way for services to say that they could not do anything for them – a ‘dustbin’ label.

Hope and the possibility of change. Feelings of hope were related to the treatment a service user was offered. Inevitably if they were told that they were untreatable this

Experience of care

led to a loss of hope and a negative outlook. The name of the disorder itself suggested a permanency, and service users questioned the use of the 'label' itself as a result, feeling that different terminology could engender more hope. Service users also found that they gained most support and hope from people they could trust and who treated them as a person and not as a diagnosis/label. For some these relationships led to a position where they felt able to question the diagnosis.

Summary. Horn and colleagues (2007) suggest that clinicians need to be aware of and sensitive to the impact of the diagnosis; clinicians should engage in discussion about the diagnosis and focus upon what may be useful to the individual user; clinical interactions should be characterised by trust and acceptance; service users should have clear communications about what 'borderline personality disorder' means; and service users should receive the message that people do move on from this diagnosis. Finally, clinicians should listen to users' own descriptions of their difficulties.

In a study by Crawford and colleagues (2007) diagnosis caused service users to have mixed views, largely due to the implications for accessing services. Many service users reported being denied services because of the diagnosis. Some felt that the terminology used was negative (having a 'disordered personality'), that stigma was attached to the diagnosis, and that they were stereotyped and judged by doctors. Some service users thought it was unfair to be labelled with such a derogatory term when they felt that the disorder had developed due to abuse at the hands of others – diagnosis made them feel like victims again. Others felt quite sceptical about the diagnosis having received a number of different diagnoses during their history of accessing services.

However, some service users welcomed the diagnosis, feeling that the symptoms fitted them quite well, and feeling some relief at having a label they could identify with. Service users were more positive about the diagnosis where the services they were accessing had a positive approach to the disorder and where they had gained a sense of shared identity with other service users (Crawford *et al.*, 2007).

In a study by Haigh (2002), which summarised the thoughts of fourteen service users on services for people with personality disorder in south England and the Midlands, people with personality disorder tended to feel labelled by society as well as by professionals after receiving the diagnosis. There was a feeling that many professionals did not really understand the diagnosis, instead equating it with untreatability. Other professionals did not disclose the diagnosis to the service user. Once the diagnosis was recorded, service users felt that the 'label' remained indefinitely and often felt excluded from services as a result. They described having the label as being the 'patients psychiatrists dislike' and felt that they were being blamed for the condition. For others, though, receiving the label was a useful experience, giving some legitimacy to their experience and helping them begin to understand themselves. Many felt that there was little clear information available about the diagnosis.

In a study by Ramon and colleagues (2001) of 50 people with personality disorder from Essex, the meaning of the term revealed a wide range of views from 'a life sentence – untreatable – no hope', to 'haven't got a clue'. The majority felt that they

did not really know what the term meant (26%), where as 22% described it as ‘a label you get when they don’t know what else to do’ and 18% referred to the meaning ‘as being labelled as bad’. Eighteen percent referred to the diagnosis as being ‘indicative of mood swings’. Service users’ own descriptions of their problems tended to correspond with an additional diagnosis, most commonly of depression and severe anxiety (36%). Service users preferred not to use the term personality disorder and found that the diagnosis led to negative attitudes by staff across a range of agencies and a refusal of treatment. Only 20% perceived the diagnosis to have led to an improvement and better treatment. A proportion of service users also felt it would be helpful if the term ‘borderline personality disorder’ were changed.

In Nehls (1999), 30 people with borderline personality disorder were interviewed to establish what it means to live with the diagnosis. Service users reported feeling that professionals held preconceived ideas and unfavourable opinions of people with a diagnosis of borderline personality disorder. They felt that they were being labelled, rather than being diagnosed. They struggled with the ramifications of having a negative label rather than the diagnosis itself, such as it affected the delivery of mental health services and also other forms of healthcare. Most of the people felt that they were in a paradox, in that they felt that they fitted the criteria, yet experienced the diagnosis as having no beneficial purpose in guiding treatment.

Self-harm and suicide attempts were commonly reported among participants interviewed by Nehls (1999). They found the view of self-harm as manipulation to be unfair and illogical, revealing an underlying prejudice and leading to a negative response to such behaviour by clinicians. Such attitudes might mean that the reasons underlying the self-destructive behaviour are missed. Service users felt it was more productive and accurate to view self-harm as a means of controlling emotional pain and not as a deliberate attempt to control others.

In a study by Stalker and colleagues (2005), which elicited the views of ten people with a diagnosis of personality disorder and analysed the data using a grounded theory approach, half felt that the term ‘personality disorder’ was disparaging. However one male participant thought that it accurately described his problems: ‘It doesn’t particularly disturb me. I don’t see any problem because that is exactly what I suffer from – a disorder of the personality’ (Stalker *et al.*, 2005).

4.3.4 Services

Six of the included studies reported service user experience of accessing services, including specialist services, staffing issues, and of the community-based pilot services for people with personality disorder.

Access to services

In the study by Haigh (2002), there was strong agreement among service users that there were not enough services for people with personality disorder and there was a lot of negativity towards those services that were available, largely due to prejudicial staff attitudes. In addition, while service users acknowledged that the care programme

Experience of care

approach had the potential to be beneficial, their experience was that it was often not followed or was unhelpful. Service users' views often improved if they were offered a specialist personality disorder service. They felt that early intervention was crucial to preventing a major deterioration in personality disorder. Service users also felt that early intervention services held more positive attitudes towards treatability and intervention.

As people with personality disorders often present in crisis and enter the mental health service through the police and other emergency services, service users interviewed by Haigh (2002) believed that self-referral may prevent further negative and unhelpful experiences. It was also felt that immediate support, which is often needed, could be provided by a telephone service, but ideally 24-hour crisis intervention teams who had knowledge of and training in personality disorders should be available as this would reduce the need for inpatient care. As GPs were usually the initial contact for access to services, it was felt that they should receive more education about personality disorders.

People interviewed by Nehls (1999) experienced services as intentionally limited, in that some of them were on a programme that only allowed them to use hospital for 2 days a month, and that the opportunities for a dialogue with mental health professionals were also limited. When in crisis, a dialogue with someone who cares was desired by service users. The push by some services towards 'self-care' and 'helping yourself' was felt to divert attention away from what matters to people with borderline personality disorder (that is, a caring response).

Access to services may also be compromised for people from black and minority ethnic backgrounds (Geraghty & Warren, 2003; see also Chapter 8). Accessing services beyond primary care may be a protracted process. In general mental health services there has been reported a poor understanding of the needs of people from black and minority ethnic backgrounds, however a service user said that once they had entered a specialist treatment service for personality disorder, it was largely able to meet their cultural needs (Jones & Stafford, 2007).

Staffing issues

Service users interviewed by Haigh (2002) felt that staff needed to be sensitive in their handling of therapeutic relationships, particularly regarding attachment, issues of gender, sexual orientation and abuse history. Staff also needed to be consistent in their assertion of boundaries and be willing to provide a reliable time commitment to a service and the people they were treating. Service users also valued input from staff who had experienced mental health difficulties, as it was felt they had more insight. All service users thought it was important to have respect from staff, to be perceived as an individual and with intelligence, to be accepting but also challenging and to view the therapeutic relationship as a collaboration. Problems arose for service users, however, when boundaries broke down and the staff began to share their own problems with service users, and when staff failed to show respect or were disinterested in the client. It was also felt that service users could provide a useful input to clinicians' training.

In the study by Ramon and colleagues (2001) based on semi-structured interviews and a questionnaire, advocates (98%) and GPs (60%) were perceived as most helpful, and CMHTs (45%) as least supportive. Service users felt that the ideal services should be those that advocated a more humane, caring response, an out-of-hours service and a safe house, an advocate service and helpline.

Specialist services

Specialist services (and long-term treatment) were viewed by the service users interviewed by Haigh (2002) as the most effective way of treating personality disorders. Service users preferred to make their own choice about services and treatments as this was felt to increase cooperation and engagement. It was stated that where there was a lack of choice and the service user opted not to engage with the treatment, this led to service users being labelled 'non-compliant'.

An acknowledgement by clinicians that short hospital admissions may be needed on occasion would be welcomed by service users (Haigh, 2002), although with less emphasis on drug treatments. An option for respite care, whether in hospital or safe/crisis houses would reduce the need for situations that result in Mental Health Act assessments. Coercive treatments were not helpful and tended to make situations worse. Service users said they would benefit from information on treatment options and being allowed to decide for themselves what would best meet their needs.

Morant and King (2003) evaluated an outpatient service attached to a therapeutic community during its first 2 years of operation. Fifteen service users (12 women, three men), the majority of whom had a diagnosis of borderline personality disorder (86%), who had received treatment for at least 1 month at the therapeutic community, were interviewed. Most service users found leaving the therapeutic community extremely difficult, particularly the adjustment from a 24-hour structure to independent living. Problems reported included depression and anxiety, feelings of isolation and loneliness, and lack of structure. Some service users returned to dysfunctional patterns of behaviour, struggled to manage relationships with family and friends, and had difficulties in managing the practical issues such as housing and contact with mental health services. Despite this post-therapeutic 'dip', most reported finding value in attending the outpatient service, but also found it to be insufficient. Those interviewed also struggled making the move back to a CMHT due to the passive and dependent role CMHTs encourage, in contrast with the responsibility people take for their own care in the therapeutic communities. Three people were admitted as inpatients during the period covered by the study. However, service users also reported a gradual structuring of daily life and establishing a network of resources. They additionally reported that the outpatient service helped them to make the transition to independent living.

Community-based pilot services for people with personality disorder

An evaluation of 11 community-based pilot sites with dedicated services for people with a personality disorder (Crawford *et al.*, 2007) included qualitative interviews and focus groups with service users and carers. The study sought to interview seven to ten

Experience of care

service users and up to three carers and former service users from each site; six current service users formed the focus group. A number of key themes emerged that covered the entire journey through the service from the entry or 'coming in' process and assessment, through experiences of different treatments, relationships with staff and other service users, boundaries and rules, out-of-hours services, to outcomes and 'endings'.

Experiences of entering the service depended on the service they were entering, but also on the user's prior experience of services. Many felt rejected or that they had been treated badly by other services, which they attributed to the personality disorder diagnosis and the complex needs and behaviours associated with it. Consequently, many of the services users felt desperate for help and relieved to be offered a service with specialist knowledge and skilled staff. Their hopes and expectations were high, but alongside this feeling was a fear of further rejection.

Service users valued receiving clear, written information about the service, particularly where it differed from mainstream services. It was also important for service users to have a welcoming response from the service; where this was not the case the service was experienced as negative and daunting.

Those interviewed tended to find assessment difficult, traumatic and upsetting, largely because of the focus on painful past experiences and the emotions these raised. Some service users felt that this process was over-long as they had to undertake tests and questionnaires over several weeks. The availability of staff to answer questions and offer support made the process easier, especially as support was often not felt to be available outside the service.

Service users welcomed services that were flexible and accessible, and staff who were responsive to the needs of service users. Service users also valued having a range of options to choose from and access at different times such as one-to-one sessions, out-of-hours phone support, crisis beds and an open clinic. It was also important that the therapy was not time limited.

Specialist services for personality disorder can lead to a strong sense of belonging for many service users due to sharing experiences with other service users and building relationships with staff. Service users also reported that these services tended to have a more positive focus, with staff having more optimistic beliefs about an individual's capacity for change and more discussions with service users about recovery.

Most of the services offered some form of psychotherapy. While most service users found psychotherapy complex and challenging, they also found it helpful and positive. Therapists' support in helping service users engage with and address their difficulties was valued and appreciated. Psychotherapy was viewed by service users as the element of the service that brought about the most significant changes and positive outcomes for people. It allowed them to understand themselves and improve their behaviour, and provided an opportunity to practice behaviours and/or communications in a safe environment. Aspects of psychotherapy, such as the DBT skills group, allowed people to find new ways of coping and thinking about their difficulties.

Rules and boundaries were a contentious issue in many of the pilot sites. People coped with these better when they were made explicit and transparent, and were able

to be negotiated, rather than being implicit and/or forced upon them. Some of the rules were felt to be too rigid and impractical, for example, attending group therapy in order to access individual therapy, not having friendships with other service users, coming off medication before starting therapy, and various rules around self-harm, such as not being able to talk in a group until the person has stopped self-harming.

The need for out-of-hours support was a common theme raised by service users. Crises usually happened outside the hours of 9 am to 5 pm, and if people did have to access a service during a crisis outside this time, the staff often responded inappropriately. Service users felt that they needed a person-centred and responsive out-of-hours service.

Few services offered support to carers. Where they were offered, carers appreciated the educational and information-giving aspects and the support of other carers. However, carers would have liked more information about the diagnosis, suggestions for how to access help and more information about care and treatment. In addition, carers felt excluded from the service user's treatment.

It was felt that the most productive relationships were with staff who were non-judgmental, helpful, supportive, caring, genuine and 'real', positive, flexible, accessible, responsive, skilled and knowledgeable. Other valuable attributes were: treating service users as whole people rather than as a collection of symptoms; being unshockable; being honest about themselves to some degree while maintaining boundaries; treating the service user as an equal; believing in the service user's capacity for change; and consequently encouraging and supporting them to achieve their goals.

Having relationships with other service users was on the whole viewed as positive, although this depended on the service model offered. Service users found it productive to share their experiences with people, as it provided them with ideas for coping, a shared sense of identity, a social network, and helped to boost their confidence. However, these relationships were more difficult to negotiate if they spent long periods of time together and there was an imbalance between giving and receiving support.

Service users expressed much anxiety about leaving a service, which was mainly centred on being required to leave before feeling ready to do so. Service users felt that a more structured approach to 'endings' was needed, and that there should be some way of retaining a link with the service and/or service users. It was also felt that reassurance was needed that they had the opportunity to restart treatment in a service if a crisis developed. Most service users felt strongly that abrupt endings were unhelpful because there was little opportunity to prepare and to work through any issues that arose out of it.

The reports from service users suggest that nearly all of the pilot services had been beneficial to people. They improved services users' confidence, self-esteem and self-awareness. Service users also came to understand their behaviours and this frequently led to changes in behaviour (such as less self-harm and fewer A&E admissions and crises), particularly as they became better able to identify the warning signs and triggers. It was also reported that services improved service users'

Experience of care

relationships and interactions with others, particularly as a result of improved communication skills.

In addition, service users felt more assertive and independent, felt that they had learnt new coping skills including managing their anger better, were able to accept care, and were increasingly thinking about returning to work or study, or able to remain in work. Service users also felt listened to and hopeful, and in more control of their lives. However, a few service users felt that the therapy they received had been damaging and/or humiliating and distressing.

However, it should be noted that in these pilot services the majority of service users were white women. Men and people of an ethnic minority were under-represented and their inclusion could have led to a less positive experience.

4.3.5 Treatments

Two studies reported on experiences of group psychotherapy for people with borderline personality disorder and there were two on DBT.

Group psychotherapy

Hummelen and colleagues (2007) interviewed eight people with borderline personality disorder who dropped out of long-term group psychotherapy following intensive day hospital treatment. The main reasons for dropping out were: finding the transition from day hospital treatment to outpatient group therapy too difficult and having bad experiences of the previous day hospital treatment; finding group therapy too distressing – service users reported having strong negative feelings evoked in therapy and feeling that these could not be adequately contained in an outpatient setting; outpatient group therapy being insufficient because too much time elapsed between sessions; being unable to make use of the group or being unsure of how the group was meant to work; experiencing a complicated relationship with the group and having a sense of not belonging; and various aspects of the patient-therapist relationship being negative (such as therapists not explaining adequately how the group worked, not dealing effectively with criticism and not acknowledging the patients' distress). Other service users found it too difficult combining work, study, or parenting responsibilities with therapy. Other reasons included a desire to escape from therapy and no interest in further long-term group therapy.

In Crawford and colleagues (2007) group psychotherapy was experienced by some service users as a good opportunity to share experiences with others and they valued the peer support. However, others, who would have preferred individual therapy, struggled where group therapy was the only option, particularly in understanding the way the group operated and its 'rules'.

Dialectical behaviour therapy (DBT)

Fourteen women with borderline personality disorder were interviewed to ascertain what is effective about DBT and why (Cunningham *et al.*, 2004). Participants reported

that DBT allowed them to see the disorder as a controllable part of themselves rather than something that controlled them, providing them with tools to help them deal with the illness. They reported that the individual therapy played an important part, particularly when the relationship with the therapist was viewed as non-judgemental and validating and the therapist pushed and challenged them. However, where the client felt that the therapist did not push enough or too much, the therapy seemed to become less effective. Another key component in the relationship is equality, with the client feeling that they were operating on the same level as the therapists and working towards the same goal. This equality seems to empower people to take more responsibility in their own therapy.

Skills training was seen as complimenting the individual therapy and being most effective when the skills trainers were able to help the service users apply the skills to their lives. The trainers needed to have a strong understanding of the skills themselves rather than just use the manual – the latter proved to be less effective for service users (Cunningham *et al.*, 2004).

Service users found some skills more helpful than others. ‘Self-soothe’, ‘distract’ and ‘one mindfulness’ were the skills reported as useful most commonly. The skills most used also corresponded to the skills most easily understood. The support that service users received in the skills group also proved to be valuable.

The 24-hour telephone skills coaching was valued by the service users as a means of supporting them through their crises (Cunningham *et al.*, 2004).

Service users reported that DBT had had a positive effect on their relationships in day-to-day interactions, and although problems with friends and family did not disappear, they were more manageable. Service users have also reported being less paranoid in public. Interpersonal skills were enhanced and this was believed to be as a result of the improvement in service users’ abilities to control their emotions and a reduction in self-harm. Although most service users felt that there were still areas that they had difficulty dealing with, some participants felt that their level of suffering had decreased, although for others it remained constant. Clients also expressed higher levels of hope and a desire to live more independently (Cunningham *et al.*, 2004).

In a study by Hodgetts and colleagues (2007) of five people (three women and two men) with borderline personality disorder being treated in an NHS DBT service in the south west of England, the participants reported that DBT was presented to them as the only treatment for personality disorder. This may have raised anxieties in service users about what was expected of them. While some valued the sense of structure to the treatment, others would have preferred a more tailored and flexible approach. There were also mixed feelings about the combination of individual therapy and group skills training. For one person the challenges of DBT proved too much so she left the programme. Another factor in her leaving was that she believed she was refused support from a crisis service because she was in a DBT programme. All of the clients interviewed saw the therapeutic relationship as important, valuing the collaborative working and the sharing of experiences. The group work gave a sense of shared identity. The participants in the group all commented on how DBT had affected them; one said that he cut himself less; others were not sure if changes in their lives were

Experience of care

due to DBT or other factors. One person was concerned that now that the option of self-harm had been removed, they had no other ‘coping’ mechanisms.

4.3.6 Personal coping strategies

One study by Stalker and colleagues (2005) reported on personal coping strategies. Participants in the survey recognised a number of strategies they employed to help them cope, the most common of which were: visiting a mental health resource centre; talking to a professional or a partner; keeping active; doing exercise; going to bed; medication; ‘keeping yourself to yourself’; ‘fighting the illness’; use of drugs and alcohol; overdosing; and cutting. The participants were fully aware that some of these activities were harmful, but felt they had no alternatives: ‘When I am feeling really bad, [drinking is] the only thing that really blots out the memories’ (Stalker *et al.*, 2005).

4.3.7 Public awareness and education

One study by Haigh (2002) reported on public awareness and education about personality disorder. It was felt by service users that more education about mental health difficulties should be provided in schools to reduce stigma, to educate about vulnerability and to teach students how to seek appropriate help if they experienced difficulties. Leaflets in GP surgeries and support groups for families/carers were also suggested. Service users also felt that it was important that people became aware that a diagnosis of personality disorder ‘doesn’t mean you’re not a nice person’.

4.3.8 Summary of helpful and unhelpful features

Helpful features identified by service users (Haigh, 2002) included: early intervention before crisis point; specialist services; choice of treatment options; care tailored to the individual; therapeutic optimism and high expectations; developing service users’ skills; fostering the use of creativity; respecting a service user’s strengths and weaknesses; clear communication; staff that were accepting, reliable and consistent; supportive peer networks; shared understanding of boundaries; appropriate follow-up and care; and making use of service users as experts in developing services and staff training.

Unhelpful features noted by service users (Haigh, 2002) included: availability of services determined by postcode; services only operating in office hours; lack of continuity in staff; staff without appropriate training; treatment decided only by diagnosis and/or funding; inability to fulfil promises made; staff that were critical of service users’ expressed needs; staff only responding to behaviour; negative staff attitudes; rigid adherence to a therapeutic model even when it is unhelpful; long-term admissions; use of physical restraint, obtrusive levels of observation, inappropriate use of medication, and withdrawal of contact used as sanction.

According to service users interviewed by Haigh (2002), services could be improved if: professionals acknowledged that personality disorder is treatable; they received a more positive experience on initial referral as this would make engagement with a service more likely; if the ending of a therapeutic relationship was addressed adequately; and if services were not removed as soon as people showed any signs of improvement, because this tended to increase anxiety and discourage maintenance of any improvement.

4.4 FAMILY AND CARER EXPERIENCE

4.4.1 Introduction

When a person is diagnosed with borderline personality disorder, the effect of the diagnosis on families and carers is often overlooked. However, a recent study has shown that psychological distress among the families and friends of people with borderline personality disorder has been likened to the distress experienced by carers of people with schizophrenia (Scheirs & Bok, 2007).

The use of the term ‘family’ in the literature generally refers to parents, siblings, spouses and children. This guideline uses the term ‘family/carer’ to apply to all people who have regular close contact with the person.

A systematic search for literature on family/carer needs, including interventions, was not undertaken on the advice of the GDG since little empirical research exists. This section therefore gives a narrative review of the available evidence and expert consensus views.

4.4.2 Do the families/carers of people with a borderline personality disorder have specific care needs?

It has been suggested (expert opinion) that families of people with a borderline personality disorder could experience what Hoffman and colleagues (2005) have described as ‘surplus stigma’, which is stigma over and above that experienced by families/carers of people with other mental illnesses. Unfortunately, there is scant empirical evidence available to support or refute this hypothesis.

Scheirs and Bok (2007) administered the Symptom Check List-90 (SCL-90) to 64 individuals biologically related (parents or siblings) or biologically unrelated (spouses or friends) to people with borderline personality disorder. The group had higher scores on all symptom dimensions of the SCL-90 than the general population. There was no significant difference between those who were biologically related to the person with borderline personality disorder and those who were not.

Hoffman and colleagues (2005) assessed burden, depression, guilt and mastery in families of people with borderline personality disorder. Forty four participants (representing 34 families) participated in a Family Connections programme (the

Experience of care

outcome of this study is described in section 4.4.3) and found significant burden as measured by the Burden Assessment Scale and Perceived Burden Scale, significant depression as measured by the Revised Centre for Epidemiological Studies Depression Scale, significant grief as measured by a Grief Scale, and low levels of mastery as measured on the Mastery Scale. It is important to note that there was significant variation in scores. This study was replicated by Hoffman and colleagues (2007b) with 55 participants who found that mean scores on the measures of burden, guilt and depression were consistent with those in the previous study.

Families/carers of people with borderline personality may have needs that are at least equivalent to families/carers of people with other severe and enduring mental health problems.

4.4.3 What intervention/support is helpful to families/carers of people with borderline personality disorder?

No RCTs of interventions specifically aimed at families/carers of people with borderline personality disorder were identified from the search for RCTs described elsewhere in this guideline, and an additional systematic search was not undertaken on the advice of the GDG. There was therefore little empirical evidence to review.

Interventions for families of people with borderline personality disorder have been strongly influenced by the literature drawn from family intervention treatments for other disorders (for example, schizophrenia). This literature has indicated that carers find psychoeducation and information most helpful (Dixon *et al.*, 2001).

However, research by Hoffman and colleagues (2003) provides a note of caution to those who advocate interventions of this type. They assessed 32 family members for their knowledge of borderline personality disorder. Knowledge was then correlated with family burden, depression and expressed emotions. Contrary to expectations greater knowledge about borderline personality disorder was associated with higher levels of burden, depression, distress and hostility towards the person with the disorder.

Berkowitz and Gunderson (2002) have piloted a multi-family treatment programme strongly influenced by psychoeducative approaches used in schizophrenia. However no outcome data were reported.

Hoffman and colleagues (2005) conducted a study examining the impact of the Family Connections programme, which aims to reduce burden, grief, depression and enhance mastery in families of people with borderline personality disorder. The programme is a 12-week manualised education programme that is strongly influenced by DBT principles. The programme also had a strong educational component in which information is provided about borderline personality disorder and research. There is a great emphasis on learning new skills (coping and family skills) and the programme aimed to foster social support. This study had 44 participants (34 families) and the families were evaluated pre-intervention, post-intervention and at 6 months follow-

up. Participants showed reductions in burden, grief and enhanced mastery. There was no significant difference in depression. The results were maintained at follow-up.

Hoffman and colleagues (2007b) was a replication of the 2005 study. Fifty five participants took part in this programme. They were assessed using the same measures as the 2005 study: pre- and post-intervention and at 3 months' follow-up (rather than 6 months in the previous study). As in the previous study, participants showed improvements in grief, burden and mastery. There was also a significant reduction in depression. While these findings are of interest and this intervention shows promise, clinical trials examining the effectiveness of this intervention have not yet been published.

There is a lack of high quality empirical evidence on interventions for families/carers of people with borderline personality disorder, although emerging evidence suggests that structured family programmes may be helpful. Hoffman and colleagues' (2003) study provides a cautionary note about giving information. Their findings suggested that more information alone could be associated with more distress.

4.4.4 Do families/carers through their behaviours and styles of relating influence clinical and social outcomes or the well-being of people with borderline personality disorder?

This clinical question needs to be explored sensitively. Families/carers could have understandable concerns with respect to this question and may feel that they are being unfairly blamed for the person's problems.

Earlier chapters (see Chapter 2) have highlighted the high correlation between childhood adversity and borderline personality disorder. These findings are challenging to families caring for people with borderline personality and it is important not to assume that all family environments are 'toxic' and have 'caused' the disorder.

There are some studies suggesting that the current family environment could influence the course of borderline personality disorder. Gunderson and colleagues (2006) explored predictors of outcome in borderline personality disorder. In this study 160 patients were recruited and followed up for 2 years at 6, 12 and 24 months. Findings should be interpreted with caution because of the nature of the measures used. However, they concluded that alongside baseline psychopathology and history of childhood trauma, present relationships was also a predictor of outcome after 2 years. The longitudinal Interval Follow-Up Evaluation was used to assess impairment in relationships with parents, spouse, siblings and children.

A significant amount of research into the impact of the family environment has focused on parental hostility and involvement and the course of a disorder. These constructs are components of expressed emotion. Expressed emotion and its impact on recovery for people with schizophrenia has been more extensively researched (see Dixon and colleagues [2001] for review). Within the borderline personality disorder literature there was only one study on expressed emotion.

Experience of care

Hooley and Hoffman (1999) followed a group of 35 people with borderline personality disorder for 1 year post-discharge. They assessed expressed emotion using the Camberwell family interview. They found no association between hostility and criticism and re-admission rates in borderline personality disorder. Even more surprising, and contrary to research in psychosis, was that people with borderline personality disorder had fewer admissions in families that scored higher on expressed over-involvement.

In summary, there is not enough evidence to confidently answer this question. It appears that the relationship between the family environment and the prognosis of borderline personality disorder is complex and multi-dimensional (Lefley, 2005). There is some tentative evidence that families of people with borderline personality disorder could interact in ways that are unhelpful for the person with borderline personality disorder. However, Lefley (2005) cautions against overly blaming families and suggests that the literature does not fully consider temperamental vulnerabilities in people with borderline personality disorder.

4.4.5 Are there interventions/support for families/carers of people with borderline personality disorder that are helpful in altering social outcome and well-being of a person with borderline personality disorder?

There are no empirical studies to review in this section. The literature is restricted to expert opinion and consensus.

4.4.6 Overall clinical summary

There is little evidence to answer clinical questions relating to support for families/carers, although families/carers of people with borderline personality disorder appear to have significant needs. Consequently, it would not be prudent to make robust clinical recommendations. Further research is needed to build on the emerging evidence suggesting that structured psychoeducation programmes that also facilitate social support networks may be helpful for families. There is an absence of research into whether family interventions alter the social outcome and welfare of a person with borderline personality disorder.

4.5 SUMMARY OF THEMES

The personal accounts and the literature reveal that during its course, borderline personality disorder can be experienced as extremely debilitating. People with the disorder report having difficulty controlling their mood, problems with relationships, an unstable sense of self, and difficulty in recognising, understanding, tolerating and

communicating emotions, which can lead to the use of coping mechanisms such as self-harm. When assessing people with borderline personality disorder it is important to recognise that physical expressions, such as self-harm, are usually indicative of internal emotions.

People with borderline personality disorder have reported that they fear rejection on entering a service, particularly if they have had prior negative experiences, and although they feel desperate for help, this can make engaging in an assessment more difficult. Assessments can be traumatic and upsetting, due in large part to the focus on painful past experiences. Explanation about the process, clear, written information about a service, and the opportunity to ask questions were all welcomed and valued.

People have reported that being diagnosed with borderline personality disorder can be both a positive and negative experience. For some it can provide a focus, a sense of control, a feeling of relief, and a degree of legitimacy to their experience. In general, people are more positive about the diagnosis when it has led to accessing services, and where those services have taken a positive approach to the disorder. However, for others, the diagnosis was equated with a loss of hope and there were reports of being denied services because of the diagnosis and associated misconceptions about its untreatability. Little information or explanation appears to be given with this diagnosis, and where it has been given it has tended to be negative. There was a feeling that different terminology, other than 'borderline personality disorder', could engender more hope. Both the personal accounts and the literature demonstrate that the diagnosis can provoke negative attitudes in healthcare professionals across a range of services and lead to a refusal of treatment.

Both the personal accounts and the qualitative literature highlight the need for healthcare professionals to be aware of the stigma surrounding borderline personality disorder and to be sensitive to the impact of the diagnosis on a person's life and their sense of hope for the future.

There is a general consensus from the literature that there are not enough services for people with personality disorder (and clinicians should be aware that access to services may be compromised for people from black and minority ethnic backgrounds). Service users felt that specialist services are most effective in treating personality disorders and that it is important to recognise that treatment may need to be long term. Early intervention was considered crucial in preventing a major deterioration in the disorder, and having the option to self-refer could prevent further unhelpful and negative experiences.

When working with people with borderline personality disorder, it was felt that healthcare professionals need to establish a collaborative partnership with the service user that is non-judgemental, supportive, caring, genuine and positive, and that they should believe in their capacity to change and encourage and support them to achieve their goals. Healthcare professionals also need to be sensitive in their handling of the therapeutic relationship, particularly regarding issues of attachment, sexual orientation and abuse history. They need to be consistent in their assertion of boundaries and willing to provide a time commitment to clients.

Experience of care

When in crisis, people felt that access to an out-of-hours crisis service was needed; a person-centred response from someone who cares and had knowledge of the disorder was felt to preferable. Working with service users to explore potential triggers for crises and strategies for managing these is useful as part of a care plan that also includes crisis advice.

Being able to have a choice about services and treatment was also important as this was felt to increase the service user's cooperation and engagement. Where this choice was lacking and the service user opted not to engage with a particular treatment this was often felt to lead to being labelled as non-compliant. Service users' own judgement about suitability or unsuitability of a service or treatment should be respected.

Service users felt that specialist personality disorder services were helpful in improving their self-esteem, self-awareness, and their understanding of their behaviour, which in turn led to a change in their behaviours (for example, a reduction in self-harm). These services also helped to improve their relationships, enabling them to feel more assertive and independent. They had established new coping skills and felt better able to accept care. However, where this service included a residential component, a 'post-therapeutic dip' was often reported as people adjusted to independent living.

Most of the services offered some form of psychotherapy, which although complex and challenging, was experienced as helpful and positive. Group psychotherapy was viewed as a good opportunity to share experiences with others and obtain peer support, although for some they would prefer individual therapy, as they found the group too distressing. This highlights the importance of how treatments can differ for individuals and the importance of client choice.

Service users have been positive about DBT because it has helped them to improve their relationships and their ability to control their emotions and reduce self-harm. However, while some valued the structure of the approach, others preferred the programme to be more tailored and flexible.

Leaving a treatment or service is often difficult for people with borderline personality disorder and can evoke strong emotions as they may feel rejected. It has been recognised that a more structured approach to 'endings' is needed. People also felt they would like reassurance that they could access the service again in a crisis. Information about support groups, activity groups and self-management techniques may also be useful.

Few services offer support to families/carers despite research that demonstrates that psychological distress in families and friends of people with borderline personality disorder is similar to that experienced by families/carers of people with schizophrenia, and they score highly on scales measuring burden and depression.

Where support is offered it tends to be centred on provision of education and information. Families/carers would like more information around the diagnosis, suggestions on how to access help and more information about care and treatment. However, there is a warning note that greater knowledge about borderline personality

disorder could increase family/carers distress. Most families/carers reported feeling excluded from the service user's treatment.

There is evidence to suggest a correlation between childhood adversity and borderline personality disorder, and that a service user's current family environment could influence the course of the disorder. However, despite this evidence it is important not to assume that all family environments are 'toxic' and have 'caused' the disorder because families/carers could feel unfairly blamed for the service user's difficulties. Collaborating with families/carers (when the service user is in agreement) and supporting them could provide a valuable resource for the person with borderline personality disorder.

4.6 CLINICAL PRACTICE RECOMMENDATIONS

4.6.1 Access to services

4.6.1.1 People with borderline personality disorder should not be excluded from any health or social care service because of their diagnosis or because they have self-harmed.

4.6.2 Developing an optimistic and trusting relationship

- 4.6.2.1 When working with people with borderline personality disorder:
- explore treatment options in an atmosphere of hope and optimism, explaining that recovery is possible and attainable
 - build a trusting relationship, work in an open, engaging and nonjudgemental manner, and be consistent and reliable
 - bear in mind when providing services that many people will have experienced rejection, abuse and trauma, and encountered stigma often associated with self-harm and borderline personality disorder.

4.6.3 Involving family/carers

- 4.6.3.1 Ask directly whether the person with borderline personality disorder wants their family or carers to be involved in their care, and, subject to the person's consent and rights to confidentiality:
- encourage family or carers to be involved
 - ensure that the involvement of families or carers does not lead to withdrawal of, or lack of access to, services
 - inform families or carers about local support groups for families or carers, if these exist.

4.6.4 Principles for healthcare professionals undertaking assessment

- 4.6.4.1 When assessing a person with borderline personality disorder:
- explain clearly the process of assessment
 - use non-technical language whenever possible
 - explain the diagnosis and the use and meaning of the term borderline personality disorder
 - offer post-assessment support, particularly if sensitive issues, such as childhood trauma, have been discussed.

4.6.5 Managing endings and transitions

- 4.6.5.1 Anticipate that withdrawal and ending of treatments or services, and transition from one service to another, may evoke strong emotions and reactions in people with borderline personality disorder. Ensure that:
- such changes are discussed carefully beforehand with the person (and their family or carers if appropriate) and are structured and phased
 - the care plan supports effective collaboration with other care providers during endings and transitions, and includes the opportunity to access services in times of crisis
 - when referring a person for assessment in other services (including for psychological treatment), they are supported during the referral period and arrangements for support are agreed beforehand with them.

5. PSYCHOLOGICAL AND PSYCHOSOCIAL TREATMENTS IN THE MANAGEMENT OF BORDERLINE PERSONALITY DISORDER

5.1 INTRODUCTION

5.1.1 Classification of therapies

Psychosocial interventions designed to help people with borderline personality disorder cover a wide range of approaches, all of which are ‘talking treatments’ but which differ in intensity, complexity and method (for example, brief psychoeducational approaches, once-weekly psychological therapy sessions and structured programmes of treatment). This chapter reviews brief psychological interventions, individual psychological therapies, psychological therapy programmes, arts therapies, complementary therapies and therapeutic communities. In addition, data from RCTs, where they exist, are analysed by outcome across all therapies.

Besides arts therapies, complementary therapies and therapeutic communities, the GDG and review team delineated three broad classes of psychological therapies: first, brief psychological interventions, which were defined as low-intensity interventions given for less than 6 months; second, individual psychological therapies, usually offered weekly but sometimes twice-weekly, in an outpatient setting (individual psychological interventions can also be configured in different ways, including standard interventions and brief interventions, and these are reported separately); third, psychological therapy programmes that combine more than one treatment (for example, individual therapy plus group therapy) (Campbell *et al.*, 2000) and are delivered by more than one therapist. More detailed descriptions of the therapies are given in the relevant sections below.

5.1.2 Delivery of psychological interventions

The method of delivery of psychological interventions has an important impact on their effectiveness. Unlike pharmacological treatments, where prescribers are assured of the quality of the product by manufacturers, the quality of a psychological intervention depends on therapists having the skills and the organisational support to replicate the intervention found effective in research settings. The levels of training and supervision of therapists and their adherence and competence in therapy delivery are carefully monitored during research trials, but rarely in NHS practice. The translation of results from trials into routine clinical practice therefore depends on NHS Trusts being aware of these quality control issues and taking steps to ensure the

Psychological and psychosocial treatments

interventions are appropriately delivered and outcomes monitored under clinical governance processes.

Typically, psychological interventions for people with borderline personality disorder are delivered by psychologists, psychiatrists, nurses and other mental health professionals with advanced training in the method being implemented and who receive regular specialist supervision. For example, therapists in DBT trials are usually doctoral or masters level professionals, have demonstrated competence in six or eight cases before being accepted and receive weekly supervision. Treatment fidelity is monitored through video or audiotape ratings. Mentalisation-based partial hospitalisation differs in that mental health staff do not hold formal qualifications, but they are trained in the method by a specialist consultant and receive twice-weekly supervision.

5.1.3 Issues in undertaking trials in patients with borderline personality disorder

There is no agreement on what constitutes the ‘core’ problem in borderline personality disorder. As the diagnosis merely requires five out of nine operational criteria to be present there are many different ways to qualify for the diagnosis, resulting in considerable heterogeneity among trial populations. This heterogeneity and variation in severity is compounded by frequent co-occurrence of other personality and axis 1 disorders, the detail of which is often not reported.

A related difficulty is in choice of outcome measures, as different treatments target specific problems and use measures designed to capture a specific outcome. For example, a common outcome measured is the incidence of deliberate self-harm, but only some people with borderline personality disorder harm themselves. The same applies to other outcomes, such as impulsivity and hostility. More universal symptom measures (such as depression) have broader applicability but are less specific to borderline personality disorder *per se*. Alternatively, pragmatic trials may measure variables related to service usage such as hospitalisation or health-related quality of life.

A challenge in conducting trials, and an important issue in developing clinically effective treatment models, is to engage and retain a representative sample of people with borderline personality disorder, since disengagement with services is common and high attrition rates from trials are usual.

5.1.4 Issues in reviewing the efficacy of psychological therapy for borderline personality disorder

The issues reviewed above have considerable implications for reviewing efficacy of treatments in borderline personality disorder, including psychological therapies. The

heterogeneity of the population samples and the outcome measures makes it difficult to combine studies and to generalise across borderline personality disorder as a whole.

Some trials have been conducted on therapies for people with borderline personality disorder aiming to modify the specific features of the disorder, whereas others included these patients in treatments for depression or anxiety. Where it is possible to extract data on the borderline personality disorder sample separately they may provide useful information, but the outcomes will inevitably be more generic.

Allegiance effects are a potential problem in interpreting results from trials. Understandably, most initial research on specific therapies is conducted by ‘product champions’—the originators of the treatment or enthusiastic followers—and almost invariably effect sizes are reported that may seldom again be demonstrated. This is probably a consequence of several factors: (a) small trials in one centre tend to create greater effect sizes than larger multicentre trials; (b) the originators may deliver the intervention more skilfully than the comparative intervention or than when replicated by others; (c) the initial collaborators also tend to be enthusiastic and more energetic in the face of difficulties so benefits are greater than when the treatment becomes standard therapy; and (d) there is scope for bias, whether conscious or not, which may exaggerate differences between the new treatment and existing ones, to emphasise the novelty of the new intervention. These factors need to be acknowledged when interpreting the results of studies. Although there is no reason to suggest that such research is itself of poorer quality, there is enough evidence that those with an allegiance to one form of therapy are more likely to find positive results for their method than independent investigators (Luborsky *et al.*, 1999) to recommend that independent studies be conducted.

5.1.5 Reviewing the evidence base

In order to make recommendations about specific psychological therapies for people with borderline personality disorder the GDG asked the clinical question:

For people with borderline personality disorder which treatments are associated with improvement in mental state and quality of life, reduction in self-harm, service use, and risk-related behaviour, and/or improved social and personal functioning while minimising harms?

The most appropriate research design to answer this is the RCT, therefore the evidence base reviewed comprised all available RCTs undertaken in people with a diagnosis of borderline personality disorder.

Summary study characteristics and descriptions of the studies are given in tables below but more information is available in Appendix 16. Similarly, summary evidence profiles are given in tables below with the full profiles in Appendix 18 and the forest plots in Appendix 17.

5.1.6 Evidence search

Psychological and psychosocial treatments

Searching for RCTs

Both published and unpublished RCTs were sought. A search was undertaken for all RCTs in people with borderline personality disorder regardless of the intervention.

Table 6: Databases searched and inclusion/exclusion criteria for RCTs of psychological and psychosocial treatments

Electronic databases	MEDLINE, EMBASE, PsycINFO
Date searched	Database inception to January 2007
Update searches	July 2007, January 2008, April 2008
Study design	RCT
Patient population	People with a diagnosis of borderline personality disorder according to DSM or similar criteria
Treatments	Any psychological or psychosocial therapy for people with borderline personality disorder as defined above
Outcomes	See below

Those for psychological and psychosocial therapies were separated from those for pharmacological interventions, which are considered elsewhere. The electronic databases searched are given in Table 6. Details of the search strings used are in Appendix 7.

Nineteen RCTs were found from searches of electronic databases and all were of brief psychological therapies, individual psychological therapies or psychological therapy programmes. One was excluded because it was found not to be randomised when the paper copy was retrieved (BOHUS2004⁶). A further two were analysed separately since they were undertaken in substance-dependent populations (LINEHAN1999; LINEHAN2002). There was one three-armed trial. Four further trials that included participants with borderline personality disorder among others, but did not report results separately, were also excluded at this stage (ABBASS2008; HUBAND2007; JOYCE2007; SPRINGER1996). Seven of the remaining trials were of DBT, but there were also trials of other cognitive behavioural therapies and psychodynamically oriented therapies (see Table 7). In addition, four RCTs of combination therapy (that is, an individual psychological therapy or psychological therapy programme added to a pharmacological treatment) were found.

In addition, the GDG contacted known researchers working on relevant trials for which pre-publication data may be available or which were likely to be published

⁶ Here and elsewhere reviewed studies are referred to by a study identifier made up of the first author's name in capital letters and date of the earliest publication relating to the study. All references relevant to a study identifier are in Appendix 16. ⁷Not an RCT.

while the guideline was being developed. This yielded seven studies: one on mentalisation-based therapy (MBT) (ANDREA unpublished⁷); one on systems training for emotional predictability and problem solving (STEPPS) (BLUM2008);

Psychological therapy programmes	
CTs	
(3)	† CARTER unpublished 2002 2000 TURNER VAN DEN BOSCH
DBT	
(8)	
TAU	
(1)	or modified psychodynamic supportive Waitlist control TAU
(4)-(6)	TAU

Brief psychological Individual psychological therapies	
Study IDs(1) TYRER2003(1) BLUM2008(1) BATEMAN2004(1) WEINBERG2006(2) CHANEN2008(2) CLARKIN [†] CLARKIN DAVIDSON2006(4) KOONS GIESEN- BLOO2006(5) LINEHAN (3) MUNROE-BLUM1995(6) LINEHAN	
Treatment(1)–(2) MACT(1) STEPPS(1) Mentalisation/day hospital CBT	(2) Transference-focused psychotherapy CBT
	(4) Schema-focused therapy Interpersonal group therapy
usual (TAU)(2) Good clinical care (manualised (2) Transference-focused psychotherapy intervention) DBT or modified psychodynamicpsychotherapy supportive psychotherapy TAU	(4) Transference-focused psychotherapy(7) Client-centred therapy Individual psychotherapy

Table 7:Included RCTs of psychological therapies

	therapies
Total participants	No. trials 2 RCTs (100)6 RCTs (708)
	Comparator(1)–(2) Treatment as (1) TAU

† armed trial, therefore appears in two columns.

Psychological and psychosocial treatments

two on DBT (CARTER unpublished⁷; FEIGENBAUM unpublished); one on CAT (CHANEN2008); and two on CT (COTTRAUX unpublished; GREGORY2008). Follow-up data from published trials (CLARKIN2004 and BATEMAN1999) were also identified. The trial by GREGORY2008 was excluded because the population had comorbid alcohol dependence and ANDREA unpublished because it was not an RCT. Two of the unpublished studies were not included to avoid compromising future publication (COTTRAUX unpublished⁸, FEIGENBAUM unpublished). Three of these trials were therefore included (BLUM2008; CARTER unpublished; CHANEN2008).

No RCTs of arts therapies, complementary therapies or therapeutic communities were found and separate searches were undertaken to identify primary research trials of any design for each of these topics. Details of these are in the relevant sections below. In addition, a search for non-RCTs of psychological therapies was undertaken since there were relatively few RCTs of psychological therapies, particularly in some of the more recently developed therapies. This is described below rather than in the relevant sections to avoid duplication.

Searching for non-randomised controlled trials in psychological therapies

Both published and unpublished non-randomised trials were sought. The electronic databases searched are given in Table 8. Details of the search strings used are in Appendix 7.

In addition, the citations excluded during the search for RCTs (above) were re-sifted to ensure that all relevant studies had been included. Non-RCTs were synthesised in narrative reviews.

Twenty non-randomised studies of either individual psychological therapies or psychological therapy programmes were found; these are listed in Table 9. An unpublished study was also made available to the GDG.

Table 8: Databases searched and inclusion/exclusion criteria for non-RCTs of psychological treatments

Electronic databases	MEDLINE, EMBASE, PsycINFO
Date searched	Database inception to October 2007
Update search	April 2008
Study design	Any non-randomised trial
Patient population	People with a diagnosis of borderline personality disorder according to DSM or similar criteria

⁷ Trial report was made available.

⁸ The trial by COTTRAUX would have been excluded because the raters were not blinded.

Treatments	Any psychological therapy for people with borderline personality disorder as defined above
Outcomes	See below

Table 9: Non-randomised studies of psychological interventions

	Individual psychological therapies	Psychological therapy programmes
No. trials (Total participants)	13 non-randomised trials (638)	8 non-randomised studies (397)
Study IDs	(1) BELLINO2005 (2) BLUM2002 (3) BROWN2004 (4) CLARKIN2001 (5) GABBARD2000 (6) HENGVELD1996 (7) LEICHSENRING2007 (8) LOFFLER-STASTKA2003 (9) LOPEZ2004 (10) MARKOWITZ2006 (11) NORDAHL2005 (12) RYLE2000 (13) WILBERG1998	(1) ALPER2001 (2) ANDREA unpublished (3) BARLEY1993 (4) CUNNINGHAM2004 (5) HARLEY2007 (6) LANIUS2003 (7) MCQUILLAN2005 (8) PRENDERGAST2007
Treatment	(1) IPT medication (2) STEPPS	(1) DBT (2) MBT

Psychological and psychosocial treatments

	(3) CT (4) Transference-focused psychotherapy (5) Psychodynamic psychotherapy (6) CBT (7) Psychoanalytically-derived therapy (8) Psychoanalytically-oriented psychotherapy (individual and group) (9) Transference-focused psychotherapy (10) IPT (11) Schema-focused therapy (12) CAT (13) Group psychotherapy	(3)–(8) DBT
Research design (comparator, if applicable)	(1) Non-randomised comparative study (2) Cohort study	(1) Case series (2) Prospective cohort study

Continued

Table 9: (Continued)

	Individual psychological therapies	Psychological therapy programmes
	(3) Uncontrolled cohort study (4)–(5) Non-comparative prospective study (6) Case series (7) Non-comparative naturalistic study (8) Unclear (9) Non-comparative prospective study (10) Abandoned RCT (11)–(12) Case series (13) Non-comparative prospective study	(3) Cohort study (4) Qualitative study of patients' views (5)–(8) Cohort study

5.1.7 Outcomes reported in RCTs

A large number of outcomes, particularly rating scales, were reported by the RCTs of psychological therapies. Those that reported sufficient data to be extractable and were not excluded are listed in Table 10. See Chapter 2 and Appendix 10 for more information on how the GDG addressed the issue of outcomes, including details of the outcomes reported by RCTs reviewed during the guideline development process.

Table 10: Outcomes extracted from psychological studies

Category	Scale
<i>Aggression</i>	Overt Aggression Scale (OAS) - aggression
<i>Anger</i>	Spielberger State-Trait Anger Expression Inventory (STAXI) Spielberger State-Trait Anger Scale (STAS)
<i>Anxiety</i>	Spielberger State-Trait Anxiety Inventory (STAI) – state anxiety and trait anxiety Hospital Anxiety and Depression Scale (HADS) – anxiety scale Hamilton Anxiety Rating Scale (HARS) Beck Anxiety Inventory (BAI)
<i>Borderline personality disorder criteria</i>	Mean number of borderline personality disorder criteria (DSM)

Continued

Table 10: (Continued)

Category	Scale
	Zanarini Rating Scale (ZAN) - borderline personality disorder
<i>Depression</i>	Hamilton Rating Scale for Depression (HRSD) Beck Depression Inventory (BDI) Montgomery Asberg Depression Rating Scale (MADRS) HADS – depression scale
<i>Drug-related</i>	Proportion of days abstinent from alcohol and drugs Proportion with clean urinalyses Mean % self-reported abstinent days (heroin)

Psychological and psychosocial treatments

<i>General functioning</i>	Global Severity Index (GSI) Global Assessment of Functioning (GAF) Global Assessment Scale (GAS) SCL-90 Clinical Outcomes in Routine Evaluation (- Outcome Measure)
<i>Mental distress</i>	GSI
<i>Hopelessness</i>	Beck Hopelessness Scale (BHS)
<i>Impulsiveness</i>	Barratt Impulsiveness Scale (BIS)
<i>Irritability</i>	OAS – irritability
<i>Quality of life</i>	World Health Organization Quality of Life Assessment (WHO QoL) European Quality of Life (Euro-QoL) Weight Health Score Value
<i>Self-harm</i>	See Table 11
<i>Service use</i>	Emergency department visits for psychiatric reasons Emergency department visits for suicidal ideation Hospital admissions for psychiatric reasons Hospital admissions for suicidal ideation Number on medication at endpoint Number with 1 inpatient admission (unspecific reasons and after self-harm) Number with 1 emergency department visit Length of psychiatric admission

Continued

Table 10: (Continued)

Category	Scale
	Length of admission following self-harm Further psychiatric outpatient treatment Number of years on 3 prescribed drugs
<i>Social functioning</i>	Social Functioning Questionnaire (SFQ) Social Problem Solving Inventory Number of years with employment
<i>Suicidality</i>	See Table 11

Acceptability	Leaving the study early for any reason
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5.1.8 Self-harm and suicide-related outcomes in the included RCTs

Self-harm and suicide-related outcomes are considered particularly important outcomes in the management of people with borderline personality disorder. They were widely reported by the RCTs of psychological therapies. However, there was considerable discrepancy among studies regarding how these were defined and reported. See Table 11 for more details.

5.1.9 Study populations

Study populations are predominantly female, particularly in trials of DBT; this is unrepresentative of men with borderline personality disorder, who are less likely to present to services, although evidence from community samples suggest that borderline personality disorder is equally prevalent in men (Singleton *et al.*, 2003). Age ranges in trials are also unrepresentative of older populations among whom modified forms of borderline personality disorder may be problematic and yet are largely unrecognised and untreated. Evidence is lacking for the effects of psychological therapies in people with borderline personality disorder from black and minority ethnic groups.

5.1.10 Potential sources of bias

Publication bias

There were too few RCTs to undertake funnel plots to ascertain publication bias so this could not be explored. However, unpublished studies were sought and included where possible.

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See Section 5.1.4 above.

	Not measured	Not measured	OAS suicidality	Not reported	Not measured	Not reported
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Self-harm acts	Suicidal acts	Published scale	Suicidal ideation
Definition: deliberate; resulted in visible tissue damage, nursing or medical intervention required	Definition: deliberate; life threatening; resulted in medical attention; medical assessment consistent with suicide attempt		Not reported
No definition given but suicidal acts reported separately	No definition given	Not used	
Not reported	Not reported		Not reported
Admission for deliberate self-harm (not defined)	Not reported		Not reported
Frequency of non-suicidal self-injury (not defined or reported separately)	Frequency of suicide attempts (not defined or reported separately)		Not used (authors' schedule available on request)
Definition: not a suicidal act; deliberate; results in potential/actual tissue damage; events occurring within 24 hours of each other considered a single act Data for number of acts given rather than number of acts per person so not extractable	Definition: deliberate; life threatening; required medical intervention (even if did not receive any) Data satisfactory as given as mean per person		Based on Acts of Deliberate Self-Harm Inventory Davidson, (

Table 11:Self-harm and suicide-related outcomes in included RCTs

Study ID	
BATEMAN1999	
BLUM2008	
CLARKIN2004	
CARTER unpublished	
CHANEN2008	
DAVIDSON2006	

Psychological and psychosocial treatments

Continued

Table 11:(Continued)

Study ID	Self-harm acts	Suicidal acts	Published scale	Suicidal ideation
GIESEN-BLOO2006 (	Reports parasuicidal subscale of the Borderline Personality Disorder Severity Index BPDSI (Linehan <i>et al.</i> , 1989) chose not to extract see Appendix	Reports parasuicidal subscale of the BPDSI (see left)	BPDSI	Reports parasui- cidity subscale of the BPDSI see left
KOONS2001 (	Reports mean number of parasuicidal acts based on Parasuicide History Interview (PHI) (Linehan <i>et al.</i> , 1989)	Reports mean number of parasuicidal acts	PHI	Beck Suicidal Ideation Scale
LINEHAN1991	Number of parasuicidal acts (unclear how defined) – acts occurring as part of one episode were counted separately in number of acts count, but number of episodes also counted	An episode that the subject considered a serious attempt to die	PHI	Used scale (self- report Scale for Suicide Ideators)

LINEHAN2006	Reports 'highest medical risk' composite measure of suicide attempts and self-injury data aggregated per year; this has not been extracted as it is not analogous to data from other studies Also reports non-suicidal injuries, which have been extracted	Also reports non-ambivalent suicide attempts it is unclear how this is defined even in the paper that reports the development of the scale)	Suicide Attempt Self-Injury Interview (non-ambivalent suicide attempts) (	Suicidal Behaviors Questionnaire Linehan & Nielsen, 1981, unpublished)
MUNROE-BLUM1995	Not reported	Not reported	Not reported	Not reported
TURNER2000	No definition given for 'rating of parasuicide'; therefore data used is 'number of suicide/self-harm attempts', which is also not defined and is self-reported	See left	Based on an unpublished scale Target Behaviour Ratings (TBR)	Beck Suicidal Ideation Scale
TYRER2003	Parasuicidal events as defined by the PHI	Not reported separately	PHI	Not reported

Continued

Table 11:(Continued)

Study ID	Self-harm acts	Suicidal acts	Published scale	Suicidal ideation
VANDENBOSCH2002	Reported as parasuicidal/self-mutilating acts using score on Lifetime Parasuicide Count (LPC) Not useable as does not give count of episodes/ acts; it is more of a composite measure of overall 'parasuicidality' in period under review	Measured as parasuicidal acts score on BPDSI which the GDG chose not to extract (see Appendix 16)	BPDSI	Not reported
WEINBERG2006	Parasuicidal events as defined by the PHI	Not reported separately	PHI	Suicidal Behaviors Questionnaire

5.1.11 Sub-analyses

Since the dataset is fairly small and there are a large number of outcomes, with different rating scales being used for the same outcome, the following sub-analyses were planned *a priori* to explore potential moderators:

Potential moderator	Sub-categories
Length of treatment	6 months versus 6 months
Manualised	Yes versus no
Number of sessions	—
Type of therapy	CBT-related versus psychodynamic-focused
Therapist experience	—
Author allegiance	—

However, since the RCTs had few outcomes in common, it was not possible to undertake these sub-analyses. Therapist experience and author allegiance are described.

5.2 ARTS THERAPIES

5.2.1 Introduction

Arts therapies developed mainly in the US and Europe during the 20th century. They have often been delivered as part of treatment programmes for people with personality disorders including those with borderline personality disorder. Four arts therapies are currently provided in Britain: art therapy, dance movement therapy, dramatherapy and music therapy. While the four different modalities use a variety of techniques and arts media, all focus on the creation of a trusting therapeutic and safe environment within which people can acknowledge and express strong emotions (Payne, 1993). These interventions are underpinned by the belief that creative processes encourage self-expression, promote self-awareness and increase insight, in the context of a reparative therapeutic relationship, thereby enhancing a person's psychological wellbeing. The creative medium in arts therapies allows the therapist to work with both verbal and non-verbal material and at different levels according to the level of disturbance in the client.

In art therapy, people are encouraged to use a range of art materials to make images and the focus is on the relationship between the image, the creator and the therapist (Waller & Gilroy, 1992; Gilroy, 2006). In dance movement therapy, therapists focus on the use of body movement and connections between mind, body and emotions are

Psychological and psychosocial treatments

explored. Dramatherapy involves creativity, play, movement, voice, storytelling, and dramatisation so that the performance arts have a central position within the therapeutic relationship. Music therapists generally co-create improvised music with talking used to guide, interpret or enhance the musical experience and its therapeutic effect (Bruscia, 1998).

Art, music and drama therapists working in the UK are state registered professions, regulated by the Health Professions Council, which requires specialist training at a masters level.

5.2.2 Databases searched and inclusion/exclusion criteria

Studies of arts therapies were sought from the citations downloaded in the search for RCTs undertaken at the beginning of the guideline development process and described above. No studies were found, so an additional search was undertaken for primary research in arts therapies in any personality disorder. Information about the databases searched and the inclusion/exclusion criteria used are in Table 12.

No studies were found from the search, therefore a general narrative review was undertaken.

5.2.3 Narrative review of arts therapies

Arts therapies have been widely used as a part of treatment programmes for people with borderline and other forms of personality disorder in Britain (Bateman & Fonagy, 1999; Haigh, 2007; Crawford *et al.*, 2007). In this context arts therapies are usually delivered in groups; individual therapy is less commonly provided. While numerous case series have described the use of arts therapies for people with borderline personality disorder (for example, Olsson & Barth, 1983; Eren *et al.*, 2000; Schmidt, 2002; Gottschalk & Boekholt, 2004; Havsteen-Franklin, 2007) very little

Table 12: Databases searched and inclusion/exclusion criteria for clinical evidence

Electronic databases	MEDLINE, EMBASE, PsycINFO, CINAHL
Date searched	Database inception to 2 May 2008
Study design	Any primary research design
Patient population	Personality disorder
Interventions	Music therapy, psychodrama, art therapy, dance therapy, writing therapies, colour therapy

Outcomes	see Table 10
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research has, so far, attempted to quantify the impact of arts therapies for people with this condition.

5.2.4 Clinical summary

There is very little research on the effectiveness of arts therapies for people with borderline personality disorder and therefore no recommendations could be made.

5.2.5 Health economic evidence

No economic evidence on arts therapies for people with borderline personality disorder was identified from the systematic search of the economic literature. Details on the methods used for the systematic review of economic literature are described in Chapter 3.

5.3 BRIEF PSYCHOLOGICAL INTERVENTIONS

5.3.1 Description of brief psychological therapy

For the purposes of this review therapy lasting less than 6 months is defined as ‘brief’. (This is distinguished from ‘time-limited’ therapies lasting more than 6 months but less than 1 year).

Manual-assisted cognitive therapy (MACT; Evans *et al.*, 1999) was developed as a public health intervention for the large numbers of people who repeatedly attempt suicide (parasuicide) rather than for borderline personality disorder *per se*. However, a high proportion of people in this population meet criteria for borderline personality disorder, and this subpopulation is therefore similar to the one for whom DBT was developed. The intervention is a brief, cognitively-oriented and problem-focused therapy comprising up to five sessions within 3 months of an episode of self-harm, with the option of a further two booster sessions within 6 months. Bibliotherapy, in the form of a 70-page booklet (Schmidt & Davidson, 2002), is used to structure the treatment sessions and to act as an *aide-memoire* between sessions. The manual covers an evaluation of the self-harm attempt, crisis skills, problem solving, basic cognitive techniques to manage emotions and negative thinking, and relapse-prevention strategies.

5.3.2 RCT evidence

Two RCTs of brief psychological therapies were found, both of MACT (TYRER2003; WEINBERG2006), with a further trial being excluded because it was in a mixed personality disorder population and data for people with borderline personality disorder were not reported separately (HUBAND2007).

5.3.3 Manual-assisted cognitive therapy

Summary study characteristics of the two trials of MACT are in Table 13 and the summary evidence profile for RCTs of MACT is in Table 14.

There is some evidence that a low-intensity intervention (MACT) has some effect on reducing self-harm and suicidal acts (reported together as a continuous measure), but no effect when reported as parasuicide as a dichotomous measure. Both these outcomes were reported by a single study, therefore it is difficult to draw firm conclusions without further research. There was no evidence of other effects on the symptoms of borderline personality disorder.

Table 13: Summary study characteristics for studies of MACT

	MACT
No. trials (Total participants)	2 RCTs (100)
Study IDs	(1) TYRER2003 (2) WEINBERG2006
N/% female	(1) 70 (borderline personality disorder group only) (2) 30/100
Mean age (or range if not given)	(1) 31 (2) 18–40
Axis I/II disorders	Borderline personality disorder
Comparator	TAU
Additional intervention	None
Setting	(1) A&E following self-harm (2) Community and outpatients
Length of treatment	(1) 3 months (2) 8 weeks

Psychological and psychosocial treatments

Length of follow-up	(1) None (2) 6 months
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acts reported togetherof parasuicide	episode General	functioning
MACT	MACT	MACT
RR 5.68	97%	0.320.17
	Moderate	Very low
1 ; n 28)	K 1 ; n 70)(	1 ; n 64)
	Psych 03.05	Psych 03.07
months4.71(11.16	—	—
	—	—
1 ; n 30)	—	—
—Psych 03.04	—	—
R 0.97 (0.88, 1.07) % versus 9%	—	—
	—	—
1 ; n 70)	—	—

Anxiety DepressionSelf-harm and suicidal No with	
MACT	
	0.42, 0.56)*(
	1 ; n 64)(
	Psych 03.01Psych 03.02Psych 03.04
	–WMD
	–Very low
	–(K
	–R
	–Moderate
	–(K
Forest plot—Psych 03.05—	

Table 14:Summary evidence profile for RCTs of MACT

Symptom	
Therapy (all versus TAU MACT unless otherwise stated)	
Clinician-rated effect size	SMD 0.5)(
	Quality of evidenceModerateVery lowModerate
Number of studies/participants	(K ₁ ; n ₆₄)(
Forest plot	
Clinician-rated effect size at follow-up 1	–
Quality of evidence	–
Number of studies/participants	–
Forest plot	–
Dichotomous data	
Quality of evidence	–
Number of studies/participants	–

*Based on skewed data.